GSSSB · STAFF NURSE EXAM

Fundamentals MCQ Bank

699 MCQs · GSSSB pattern · Answer key with explanation



20 Sections | 699 MCQs | Direct + Priority + Scenario | Match & Assertion-Reason | Calculations | Explanations

Hinglish explanation + English exam terms



Sister Asha
Aapki Nursing teacher

MCQ Bank – Sections

1. **Nursing Profession, History, Ethics, Law & Theories**
   40 MCQs · Nightingale, history, INC/TNAI/ICN, roles, ethics, legal terms, consent, theories
2. **Health, Hospital, Admission-Discharge & Patient Safety**
   27 MCQs · Health concepts, levels of care, admission, discharge, LAMA, IPSG, falls, restraints, fire, codes
3. **Communication, Health Education & Documentation**
   26 MCQs · Communication process, therapeutic techniques, NPR, health education, records, SOAP, SBAR
4. **Nursing Process & Priority Setting**
   27 MCQs · ADPIE, data types, NANDA, PES, goals, Maslow, delegation, evaluation
5. **Vital Signs**
   38 MCQs · Temperature, fever types, thermometers, pulse, respiration patterns, BP technique & errors, SpO2
6. **Health Assessment & Physical Examination**
   30 MCQs · IPPA, percussion notes, equipment, positions, skin, chest, heart, abdomen, neuro grading, BMI
7. **Infection Control, Asepsis, Sterilization & BMW**
   38 MCQs · Chain, HAI, hand hygiene, PPE, isolation, sterile field, sterilization, disinfectants, BMW, needle-stick
8. **Personal Hygiene, Bed Making & Comfort Devices**
   28 MCQs · Bath, oral care, unconscious mouth care, dentures, hair & lice, eye/ear, perineal, back care, beds, comfort devices
9. **Rest, Sleep & Pain**
   28 MCQs · Sleep stages, requirements, disorders, sleep hygiene, pain theories, types, scales, analgesics
10. **Body Mechanics, Mobility, Positions & Pressure Ulcers**
    33 MCQs · Body mechanics, ROM terms, immobility hazards, positions, transfers, crutch-cane-walker, pressure ulcer stages, Braden
11. **Nutrition & Feeding**
    30 MCQs · Nutrients, vitamins, minerals, PEM, therapeutic diets, feeding, NG tube, PEG, TPN, I/O
12. **Urinary Elimination & Catheterization**
    30 MCQs · Urine values & terms, specimens, catheter sizes & insertion, catheter care, retention, incontinence
13. **Bowel Elimination – Enema & Suppository**
    28 MCQs · Stool, constipation, impaction, laxatives, enema types, volumes, position, temperature, suppository, flatus tube, stool specimen
14. **Oxygenation & Respiratory Care**
    30 MCQs · Hypoxia signs, O2 devices & flows, cylinder, safety, nebulization, inhalers, CPT, postural drainage, suctioning, tracheostomy
15. **Fluids, IV Therapy & Blood Transfusion**
    34 MCQs · Body fluids, dehydration vs overload, IV fluids, cannula colours, flow-rate calculations, complications, transfusion

16 **Medication Administration & Drug Calculations**
40 MCQs · Rights, abbreviations, routes, injections, angles, sites, Z-track, insulin, eye/ear drops, narcotics, calculations

17 **Hot & Cold, Wound Care, Bandaging & Specimens**
30 MCQs · Heat & cold effects & contraindications, temperatures, wound healing, dressing, drains, sutures, bandage turns, specimens

18 **First Aid & Emergency Care**
34 MCQs · Principles, CPR, choking, bleeding, shock, fractures, burns, poisoning, bites, heat stroke, triage

19 **Unconscious Patient, Peri-operative, Stress, Grief & Death**
28 MCQs · Unconscious care, pre/post-op basics, GAS, defence mechanisms, Kübler-Ross, dying, signs of death, last office, organ donation

20 **GSSSB Pattern Mock Test – 100 MCQs (Full Syllabus)**
100 MCQs · 100 questions, 100 marks, 0.25 negative marking – full Fundamentals of Nursing

Level 1 – Direct: facts, values, definitions

Level 2 – Priority: "first / best / immediate" action

Level 3 – Advanced: scenario, NOT/EXCEPT, match, A-R, calculation



MCQ kaise solve karein?

Pehle khud answer socho, phir options padho. Har section ke baad answer key aur explanation hai, galti wale questions ka number apni notebook me likho. Exam me galat answer par **0.25 marks** kat sakte hain, isliye bina soche guess mat karo.

1 Nursing Profession, History, Ethics, Law & Theories

Level 1 – Direct

1. "Notes on Nursing: What it is and what it is not" kisne likhi?
   (a) Virginia Henderson (b) Clara Barton (c) Lillian Wald (d) Florence Nightingale
2. Florence Nightingale ka janm kab hua tha?
   (a) 12 May 1820 (b) 12 May 1854 (c) 13 August 1910 (d) 1 December 1820
3. Florence Nightingale ne nursing school kahan shuru kiya (1860)?
   (a) Geneva (b) St. Thomas' Hospital, London (c) Kaiserswerth (d) Scutari
4. Indian Nursing Council Act kis saal pass hua?
   (a) 1908 (b) 1926 (c) 1947 (d) 1949
5. TNAI ki sthapna kab hui?
   (a) 1899 (b) 1908 (c) 1922 (d) 1947
6. ICN (International Council of Nurses) ka headquarter kahan hai?
   (a) New Delhi (b) Geneva (c) London (d) New York
7. Nursing ki 14 basic needs kisne di?
   (a) Peplau (b) Abdellah (c) Orem (d) Virginia Henderson
8. "Self-care deficit theory" kisne di?
   (a) Callista Roy (b) Betty Neuman (c) Dorothea Orem (d) Imogene King
9. "Adaptation model" kis theorist ka hai?
   (a) Martha Rogers (b) Sister Callista Roy (c) Jean Watson (d) Myra Levine
10. "Transcultural nursing" ke founder –
    (a) Ida Orlando (b) Madeleine Leininger (c) Faye Abdellah (d) Hildegard Peplau
11. Peplau ki theory ka naam –
    (a) Goal attainment (b) Systems model (c) Interpersonal relations (d) Human caring
12. "From Novice to Expert" model kisne diya?
    (a) Lydia Hall (b) Nola Pender (c) Kolcaba (d) Patricia Benner
13. International Nurses Day kab manaya jata hai?
    (a) 7 April (b) 12 May (c) 8 May (d) 1 July
14. Nightingale Pledge kisne likha?
    (a) Ethel Fenwick (b) Florence Nightingale (c) Lystra Gretter (d) Isabel Robb
15. India me pehla hospital kahan bana?
    (a) Mumbai (b) Kolkata (c) Fort St. George, Madras (d) Delhi
16. "Father of Medicine" kise kaha jata hai?
    (a) Sushruta (b) William Osler (c) Hippocrates (d) Galen
17. "Do no harm" kis ethical principle ko darshata hai?
    (a) Justice (b) Beneficence (c) Fidelity (d) Non-maleficence
18. Patient ko sach batana kaunsa principle hai?
    (a) Veracity (b) Justice (c) Autonomy (d) Fidelity

19. Bina consent patient ko touch karna (injection dena) kehlata hai –
(a) Libel (b) Battery (c) Negligence (d) Assault

20. Likhit roop me kisi ki badnami karna –
(a) Tort (b) Libel (c) Assault (d) Slander

? Level 2 – Priority & Application

21. Patient surgery consent form sign karne se pehle bolta hai "mujhe samajh nahi aaya kya operation hoga". Nurse kya kare?
(a) Sign karwa le, doctor baad me samjha dega (b) Khud surgery ke risk samjha de (c) Relative se sign karwa le (d) Surgeon ko inform kare ki patient ko aur jaankari chahiye

22. Patient ne pre-medication (sedative) le li hai, ab consent form sign nahi hua. Sahi kadam –
(a) Consent abhi valid nahi; doctor ko inform karo (b) Surgery cancel kar do (c) Nurse khud sign kare (d) Patient se turant sign karwao

23. Nurse dawai dene se pehle patient ko kehti hai "dawa nahi li to zabardasti injection lagaungi". Ye kya hai?
(a) Negligence (b) False imprisonment (c) Assault (d) Battery

24. Confused patient ko bina doctor ke order ke restraint lagana –
(a) Defamation (b) Beneficence (c) Allowed for safety (d) False imprisonment

25. Nurse ne side rails up nahi kiye aur sedated patient gir gaya. Ye hai –
(a) Battery (b) Negligence (c) Assault (d) Libel

26. 16 saal ke patient ki elective surgery ke liye consent kaun dega?
(a) Nurse (b) Patient khud (c) Parent/ guardian (d) Doctor

27. Accident me behosh patient, koi relative nahi, emergency surgery chahiye. Consent?
(a) Implied consent – emergency me life-saving treatment (b) Surgery delay karo (c) Police ka consent (d) Nurse sign kare

28. Nurse ko doctor ka order galat lagta hai (dose zyada). Kya kare?
(a) Doctor se clarify karo, phir hi do (b) Dawai de do – doctor zimmedar hai (c) Order ignore karo (d) Aadhi dose do

29. HIV positive patient ka relative nurse se status poochta hai. Nurse –
(a) Doctor se poochne ko kehti hai, phir khud bata deti hai (b) Report dikha deti hai (c) Confidentiality maintain karti hai (d) Bata deti hai

30. Peplau ke anusar first meeting me nurse patient ka trust jeetne par focus karti hai – ye kaunsa phase hai?
(a) Orientation (b) Resolution (c) Exploitation (d) Identification

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

31. Valid informed consent ke liye kaunsi condition ZAROORI NAHI hai?
(a) Patient adult ho (b) Patient ke relative ki permission ho (c) Patient voluntarily sign kare (d) Patient mentally competent ho

32. Malpractice ke 4 elements me se kaunsa NAHI hai?
(a) Damage (b) Duty (c) Defamation (d) Breach of duty

33. Match: P. Orem Q. Roy R. King S. Neuman — with 1. Goal attainment 2. Self-care 3. Stressors/ systems 4. Adaptation
(a) P-4, Q-2, R-3, S-1 (b) P-1, Q-3, R-2, S-4 (c) P-2, Q-1, R-4, S-3 (d) P-2, Q-4, R-1, S-3

34. Match: P. Hippocrates Q. Semmelweis R. Lister S. Henry Dunant — with 1. Red Cross 2. Antiseptic surgery 3. Hand washing 4. Father of medicine
(a) P-4, Q-3, R-2, S-1 (b) P-4, Q-2, R-3, S-1 (c) P-1, Q-3, R-2, S-4 (d) P-3, Q-4, R-1, S-2

35. Assertion (A): Nurse informed consent form par witness ke roop me sign karti hai. Reason (R): Procedure samjhana nurse ki primary responsibility hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

36. Assertion (A): Orem ke supportive-educative system me nurse patient ko teach & guide karti hai. Reason (R): Is system me patient self-care khud kar sakta hai lekin guidance chahiye.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

37. Roy ke adaptation model ke 4 adaptive modes me kaunsa NAHI hai?
(a) Physiological (b) Self-actualization (c) Role function (d) Self-concept

38. Benner ke stages ka sahi kram –
(a) Beginner, Novice, Competent, Expert, Proficient (b) Novice, Competent, Advanced beginner, Proficient, Expert (c) Novice, Advanced beginner, Competent, Proficient, Expert (d) Novice, Proficient, Competent, Advanced beginner, Expert

39. Nursing ke metaparadigm me kaunsa concept NAHI hai?
(a) Person (b) Hospital (c) Environment (d) Health

40. "Respondeat superior" ka matlab –
(a) Statute of limitations (b) Thing speaks for itself (c) Let the master answer – employer bhi zimmedar (d) Good Samaritan protection

Answer Key & Explanation

1-d Nightingale ne 1859 me likhi.
2-a 12 May 1820, Florence (Italy) – isliye 12 May Nurses Day.
3-b Nightingale Training School, St. Thomas' Hospital, 1860.
4-c INC Act 1947; council 1949 me bani.
5-b TNAI 1908; journal 1910 se.
6-b ICN – 1899, Geneva.
7-d Henderson – 14 basic needs.
8-c Orem – self-care deficit, 3 nursing systems.
9-b Roy – 4 adaptive modes.
10-b Leininger – sunrise model.
11-c Peplau – 4 phases of nurse-patient relationship.
12-d Benner – 5 stages (Dreyfus model).
13-b Nightingale ka birthday.
14-c Lystra Gretter, 1893, Detroit.
15-c 1664, East India Company.
16-c Hippocrates – Hippocratic oath.
17-d Non-maleficence = harm na karna.
18-a Veracity = truthfulness.
19-b Actual touching = battery; threat = assault.
20-b Libel written, slander spoken.
21-d Samjhana surgeon ka kaam; nurse advocate ban ke inform kare.
22-a Sedation ke baad consent invalid.
23-c Threat (bina touch) = assault.
24-d Unnecessary/ unordered restraint = false imprisonment.
25-b Expected care na dena = negligence.
26-c Minor (< 18) – parent/ guardian.
27-a Emergency me implied consent.
28-a Doubtful order hamesha clarify karo.
29-c Confidentiality – patient ki permission ke bina nahi.
30-a Orientation – trust & problem identification.
31-b Adult competent patient ka khud ka consent kaafi hai.
32-c Elements: duty, breach, damage, causation.
33-d Orem self-care, Roy adaptation, King goal, Neuman stressors.

34-a Hippocrates medicine, Semmelweis hand wash, Lister antisepsis, Dunant Red Cross.

35-c A sahi; samjhana doctor ki responsibility, isliye R galat.

36-a Dono sahi aur R, A ko explain karta hai.

37-b Modes: physiological, self-concept, role function, interdependence.

38-c Novice → advanced beginner → competent → proficient → expert.

39-b Metaparadigm: person, environment, health, nursing.

40-c Vicarious liability – employer bhi responsible.

2 Health, Hospital, Admission-Discharge & Patient Safety

Level 1 – Direct

1. WHO ki health definition kis saal di gayi?
   (a) 1946/1948 (b) 1952 (c) 1978 (d) 1986
2. World Health Day kab hai?
   (a) 7 April (b) 12 May (c) 1 December (d) 31 May
3. Discharge planning kab shuru karni chahiye?
   (a) Admission ke samay (b) Surgery ke baad (c) Doctor ke order ke baad (d) Discharge ke din
4. Patient identification ke liye minimum kitne identifiers?
   (a) 1 (b) 2 (c) 3 (d) 4
5. Morse Fall Scale me high risk score –
   (a) ≥ 25 (b) ≥ 45 (c) ≥ 18 (d) ≥ 9
6. Restraint lagane ke baad check kitni der me?
   (a) Once a shift (b) Every 15–30 min (c) Every 4 hrs (d) Every 8 hrs
7. Restraint release karke ROM/ toileting kitni der me?
   (a) Daily (b) Every 6 hrs (c) Every 2 hrs (d) Every 30 min
8. Fire safety ka sequence –
   (a) ABCD (b) SBAR (c) PASS (d) RACE
9. "Code Blue" ka matlab –
   (a) Cardiac/ respiratory arrest (b) Child abduction (c) Bomb threat (d) Fire
10. "Code Pink" ka matlab –
    (a) Mass casualty (b) Infant/ child abduction (c) Fire (d) Violent patient
11. Do beds ke beech ideal doori –
    (a) 1 ft (b) 3–6 ft (1–2 m) (c) 10 ft (d) 15 ft
12. Electrical fire me kaunsa extinguisher?
    (a) CO_2/ dry powder (b) Water (c) Foam always (d) Sand only
13. World Patient Safety Day –
    (a) 17 September (b) 7 April (c) 5 May (d) 15 October
14. Leavell & Clark ke anusar screening kis level of prevention me?
    (a) Tertiary (b) Secondary (c) Primary (d) Primordial

Level 2 – Priority & Application

15. Patient bed ke paas floor par gira mila. Nurse ka pehla kadam –
    (a) Relative ko call karo (b) Incident report bharo (c) Turant utha ke bed par lita do (d) Pehle assess karo (ABC, injury, LOC) phir help bulao
16. Patient LAMA jaana chahta hai. Nurse kya kare?
    (a) Darwaza band kar de (b) Risks samjhaye, doctor ko inform kare, LAMA form sign karwaye (c) Bill bharne tak roke (d) Security bulaye

17. Restraint ko kahan bandhna chahiye?
(a) Bed frame ke movable part se (b) Chair se (c) Side rail se (d) IV stand se

18. Fall risk patient ke liye sabse sahi intervention –
(a) Lights band (b) Charon side rails up (c) Sedative do (d) Bed lowest position, wheels lock, call bell paas

19. Hospital me aag lagi; ward me ambulatory, wheelchair aur bed-ridden patients hain. Pehle kise nikalein?
(a) Bed-ridden (b) Ambulatory (c) Sabse beemar (d) Wheelchair

20. Naya patient admit hua. Nurse ki pehli zimmedari –
(a) Bill banana (b) Doctor ka intezaar (c) Patient ko comfortable & safe karke orient karna, assess karna (d) Relatives ko bahar bhejna

21. Patient ko identify karne ka sahi tareeka –
(a) Diagnosis se pehchanna (b) "Aap Ramesh hain na?" poochhna (c) Bed number poochhna (d) Patient se naam & DOB batane ko kehna, ID band se match karna

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

22. IPSG (International Patient Safety Goals) me kaunsa NAHI hai?
(a) Daily bed bath (b) Correct patient identification (c) Hand hygiene for HAI (d) Reduce falls

23. Match: P. Code Red Q. Code Blue R. Code Pink S. Code Black — with 1. Bomb threat 2. Fire 3. Cardiac arrest 4. Infant abduction
(a) P-2, Q-3, R-4, S-1 (b) P-3, Q-2, R-1, S-4 (c) P-1, Q-3, R-4, S-2 (d) P-2, Q-4, R-3, S-1

24. Assertion (A): Restraint ko side rail se nahi bandhna chahiye. Reason (R): Side rail neeche karne par patient ko injury ho sakti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

25. Class A fire me kya jalta hai?
(a) Electrical (b) Petrol, oil (c) Wood, paper, cloth (d) Metals

26. Suchman ke illness behaviour stages me pehla stage –
(a) Recovery (b) Sick role (c) Medical care contact (d) Symptom experience

27. Discharge teaching "METHOD" me kaunsa NAHI aata?
(a) Medications (b) Hospital bill (c) Diet (d) Treatment

✓ Answer Key & Explanation

1-a WHO 1946 me bana, definition 1948 se lagu.
2-a WHO founding day, 7 April.
3-a Discharge planning admission se shuru hoti hai.
4-b Do identifiers – naam + DOB/ UHID.
5-b Morse ≥ 45 = high risk.
6-b Circulation & skin har 15–30 min check.
7-c Har 2 hrs release, ROM, toilet.
8-d Rescue, Alarm, Contain, Extinguish.
9-a Code Blue = arrest.
10-b Code Pink = infant/ child abduction.
11-b 3–6 ft cross-infection rokne ke liye.
12-a Electrical fire par paani nahi.
13-a WHO, 2019 se.
14-b Early diagnosis = secondary prevention.
15-d Move karne se pehle assess karo (spinal/ head injury ho sakti hai).
16-b Autonomy ka respect, document karo; rokna false imprisonment hai.
17-a Movable frame se – side rail neeche karne par injury nahi.
18-d Low bed, locked wheels, call bell sabse effective.
19-b Ambulatory pehle (jaldi niklenge), phir wheelchair, phir bed-ridden.
20-c Comfort, safety, orientation, assessment.
21-d Open question + ID band match.
22-a Bed bath safety goal nahi hai.

23-a Red fire, Blue arrest, Pink abduction, Black bomb.
24-a Dono sahi, R reason hai.
25-c Class A = ordinary combustibles.
26-d Symptom experience pehla stage.
27-b METHOD = Medications, Environment, Treatment, Health teaching, Outpatient, Diet.

3 Communication, Health Education & Documentation

Level 1 – Direct

1. Communication process me "feedback" kaun deta hai?
   (a) Channel (b) Sender (c) Noise (d) Receiver
2. Intimate zone ki doori –
   (a) 0–18 inch (b) 18 inch–4 ft (c) 4–12 ft (d) > 12 ft
3. SOLER me "L" ka matlab –
   (a) Lean forward (b) Laugh (c) Look (d) Listen
4. POMR/ SOAP charting kisne introduce kiya?
   (a) Lawrence Weed (b) Peplau (c) Nightingale (d) Benner
5. SOAP me "A" ka matlab –
   (a) Advice (b) Answer (c) Assessment (d) Action
6. SBAR me "R" –
   (a) Report (b) Reason (c) Record (d) Recommendation
7. Skill sikhane ka best method –
   (a) Poster (b) Lecture (c) Pamphlet (d) Demonstration & return demonstration
8. Bloom's taxonomy me attitude/ feelings kis domain me?
   (a) Cognitive (b) Social (c) Psychomotor (d) Affective
9. "Cone of experience" kisne diya?
   (a) Bloom (b) Edgar Dale (c) Maslow (d) Rogers
10. Kardex kya hai?
    (a) Permanent legal record (b) Patient care plan ka quick reference card (c) Consent form (d) Incident report
11. Peplau ka last phase –
    (a) Identification (b) Orientation (c) Working (d) Resolution
12. Record likhne ke liye ink –
    (a) Pencil (b) Green ink (c) Blue/ black permanent ink (d) Red ink

Level 2 – Priority & Application

13. Patient kehta hai "Kal meri surgery hai, mujhe bahut darr lag raha hai." Best response –
    (a) "Doctor bahut experienced hain" (b) "Aapko kis baat ka darr hai? Mujhe batayiye" (c) "Chinta mat karo, sab theek ho jayega" (d) "Darrne ki kya baat hai?"
14. Kaunsa response non-therapeutic hai?
    (a) "Mujhe aur batayiye" (b) "Aap pareshan lag rahe hain" (c) Silence rakhna (d) "Aapne dawai kyun nahi li?"
15. Nurse se chart me galat entry ho gayi. Kya kare?
    (a) White ink lagaye (b) Page phaad de (c) Single line kheench ke "error" likh ke sign kare (d) Upar overwrite kare
16. Hearing impaired patient se baat karte waqt –
    (a) Peeche se bolo (b) Bahut tezi se bolo (c) Zor se chillao (d) Saamne khade hokar, roshni me, dheere bolo

17. Visually impaired patient ke kamre me ghuste waqt nurse –
(a) Pehle apna naam batati hai, phir chhooti hai (b) Chupchaap kaam karti hai (c) Seedha injection lagati hai (d) Furniture shift karti hai

18. Unconscious patient ke paas nurse –
(a) TV on rakhti hai (b) Procedure explain karke karti hai, kyunki sunne ki shakti reh sakti hai (c) Relatives se patient ki halat discuss karti hai (d) Baat nahi karti

19. Medication error hua; incident report kahan rakhein?
(a) Hospital administration/ risk management ke paas (b) Patient ki file me (c) Patient ko de do (d) Nurses' notes me mention karo

20. Doctor ko patient ki bigadti condition phone par batani hai. Best format –
(a) Narrative (b) SOAP (c) PIE (d) SBAR

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

21. Achhi documentation ki kaunsi baat GALAT hai?
(a) Turant care ke baad (b) Factual & objective (c) Legible (d) Doosri nurse ki care bhi chart karna

22. Match: P. Restating Q. Reflecting R. Silence S. Clarifying — with 1. "Aapka matlab kya hai?" 2. "Aap gussa lag rahe hain" 3. "Aap kehna chahte hain neend nahi aati?" 4. Patient ko sochne ka time
(a) P-3, Q-2, R-4, S-1 (b) P-2, Q-3, R-1, S-4 (c) P-4, Q-2, R-3, S-1 (d) P-3, Q-1, R-4, S-2

23. Assertion (A): "Why" questions non-therapeutic hote hain. Reason (R): "Why" patient ko defensive bana deta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

24. Kaunsa abbreviation "Do Not Use" list me hai?
(a) U (units) (b) mg (c) ml (d) kg

25. Records ka kaunsa purpose NAHI hai?
(a) Communication (b) Legal evidence (c) Patient ki gossip (d) Research

26. Transference ka matlab –
(a) Patient apni purani feelings nurse par daale (b) Patient transfer (c) Report handover (d) Nurse apni feelings patient par daale

✓ Answer Key & Explanation

1-d Receiver response deta hai.
2-a Intimate 0–18 inch (physical care).
3-a Sit squarely, Open, Lean forward, Eye contact, Relaxed.
4-a Lawrence Weed, 1969.
5-c Subjective, Objective, Assessment, Plan.
6-d Situation, Background, Assessment, Recommendation.
7-d Psychomotor skill – return demonstration.
8-d Affective = attitude, values.
9-b Edgar Dale – direct experience best.
10-b Kardex = quick summary, permanent record nahi.
11-d Resolution = termination.
12-c Permanent ink, pencil nahi.
13-b Open-ended, feelings explore karna therapeutic hai.
14-d "Why" defensive banata hai.
15-c Erase/ white-out kabhi nahi.
16-d Lip reading ke liye face visible ho.
17-a Apna parichay do, phir touch karo.
18-b Hearing last sense hai.
19-a Incident report chart ka part nahi.
20-d SBAR structured handover/ communication.
21-d Sirf apni care ka chart.
22-a Restating paraphrase, reflecting feelings, silence time, clarifying meaning.
23-a Dono sahi, R reason hai.
24-a "U" ko 0 ya 4 padha ja sakta hai – "units" likho.

25-c Records confidential hain.
26-a Counter-transference nurse ki taraf se hota hai.

4 Nursing Process & Priority Setting

Level 1 – Direct

1. "Nursing process" term sabse pehle kisne use kiya?
   (a) NANDA (b) Lydia Hall (c) Orem (d) Yura & Walsh
2. Nursing process ka first step –
   (a) Evaluation (b) Planning (c) Diagnosis (d) Assessment
3. Data ka sabse reliable (primary) source –
   (a) Patient (b) Lab report (c) Family (d) Medical record
4. "Patient kehta hai sir dard ho raha hai" – ye data hai –
   (a) Subjective (b) Secondary (c) Objective (d) Tertiary
5. Kaunsa objective data hai?
   (a) BP 150/90 (b) Itching (c) Nausea (d) Anxiety
6. NANDA ka full form –
   (a) Nursing And Nutrition Diagnostic Association (b) North American Nursing Diagnosis Association (c) North Asian Nursing Diagnosis Agency (d) National Association of Nursing Diagnosis
7. PES format me "E" –
   (a) Evaluation (b) Expected outcome (c) Etiology (d) Evidence
8. "As evidenced by" kis part ko darshata hai?
   (a) Signs & symptoms (b) Goal (c) Problem (d) Etiology
9. Risk nursing diagnosis me kitne parts?
   (a) 1 (b) 2 (c) 3 (d) 4
10. Goals ka criteria –
    (a) SBAR (b) SOAP (c) PIE (d) SMART
11. Maslow ki sabse neeche wali need –
    (a) Love (b) Physiological (c) Safety (d) Esteem
12. Maslow ki sabse upar wali need –
    (a) Belonging (b) Esteem (c) Safety (d) Self-actualization

Level 2 – Priority & Application

13. Char patients me sabse pehle kise dekhenge?
    (a) Discharge ke liye ready patient (b) Diabetic jo breakfast maang raha hai (c) Fracture leg wala patient pain me (d) Stridor wala patient
14. Kaunsa goal statement sahi likha hai?
    (a) Patient will know insulin (b) Nurse will teach diet (c) Patient will demonstrate insulin self-injection correctly by day 3 (d) Patient will understand diabetes
15. Kaunsa task ANM/ nursing assistant ko delegate kar sakte hain?
    (a) Discharge teaching (b) Unstable patient ka evaluation (c) New admission assessment (d) Stable patient ka bed bath

16. Nursing diagnosis sahi likha hai –
(a) Needs suctioning (b) Ineffective airway clearance r/t thick secretions AEB crackles (c) Acute pain r/t appendicitis (d) Pneumonia r/t infection

17. Patient ka goal "not met" hua. Nurse kya kare?
(a) Reassess karke plan revise kare (b) Discharge kare (c) Patient ko blame kare (d) Plan band kar de

18. Implementation se pehle nurse kya kare?
(a) Goal likhe (b) Patient ko reassess kare (c) Document (d) Evaluation

19. Turning & positioning kis type ka intervention hai?
(a) Medical (b) Collaborative (c) Dependent (d) Independent

20. IV antibiotics dena kis type ka intervention?
(a) Autonomous (b) Independent (c) Dependent (d) Interdependent

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

21. Nursing process ki kaunsi characteristic GALAT hai?
(a) Cyclic (b) Client-centred (c) Static & rigid (d) Dynamic

22. Goal statement me kaunsa verb measurable NAHI hai?
(a) Walk (b) Understand (c) Demonstrate (d) List

23. Match: P. Assessment Q. Diagnosis R. Planning S. Evaluation — with 1. Goal met/not met 2. Data collection 3. Priority & goals 4. Clinical judgement (NANDA)
(a) P-1, Q-4, R-3, S-2 (b) P-4, Q-2, R-1, S-3 (c) P-2, Q-3, R-4, S-1 (d) P-2, Q-4, R-3, S-1

24. Assertion (A): Medical diagnosis treatment ke saath badalta nahi. Reason (R): Nursing diagnosis patient ke response par based hota hai aur badal sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

25. Delegation ke "5 rights" me kaunsa NAHI hai?
(a) Right supervision (b) Right person (c) Right salary (d) Right task

26. Gordon ne kitne functional health patterns diye?
(a) 11 (b) 14 (c) 21 (d) 7

27. Evaluation ka sabse important type –
(a) Outcome (b) Process (c) Cost (d) Structure

✓ Answer Key & Explanation

1-b Lydia Hall, 1955.
2-d Assessment – data collection.
3-a Patient primary source hai.
4-a Patient jo bataye = subjective.
5-a Measurable sign = objective.
6-b 2002 se NANDA-International.
7-c Problem, Etiology, Signs/symptoms.
8-a AEB = defining characteristics.
9-b Risk diagnosis me S nahi hota.
10-d Specific, Measurable, Achievable, Realistic, Time-bound.
11-b Physiological base par hai.
12-d Self-actualization top par.
13-d Stridor = airway – ABC priority.
14-c Measurable verb + criteria + time.
15-d Stable, routine task delegate ho sakta hai.
16-b Response + etiology + signs; medical term etiology nahi.
17-a Reassess & revise – process cyclic hai.
18-b Condition badal sakti hai, pehle reassess.
19-d Nurse khud initiate karti hai.
20-c Doctor ke order par = dependent.
21-c Process flexible & dynamic hai.
22-b "Understand" measure nahi kar sakte.
23-d Assessment data, diagnosis judgement, planning goals, evaluation outcome.

24-b Dono sahi, lekin R, A ko explain nahi karta.
25-c Salary delegation ka right nahi.
26-a Gordon – 11 patterns.
27-a Outcome (patient ka change) sabse important.

5 Vital Signs

Level 1 – Direct

1. Body ka thermoregulatory centre –
   (a) Pons (b) Cerebellum (c) Hypothalamus (d) Medulla
2. Normal oral temperature –
   (a) 36 °C (b) 37 °C (98.6 °F) (c) 38 °C (d) 39 °C
3. Hyperpyrexia ka matlab temperature –
   (a) > 38 °C (b) > 39 °C (c) > 41 °C (d) > 37.5 °C
4. Hypothermia –
   (a) < 35 °C (b) < 37 °C (c) < 36.5 °C (d) < 30 °C only
5. Sabse accurate temperature route –
   (a) Groin (b) Axillary (c) Rectal (d) Oral
6. Sabse safe route –
   (a) Tympanic (b) Axillary (c) Oral (d) Rectal
7. Axillary temperature kitni der rakhna chahiye (glass thermometer)?
   (a) 1 min (b) 3 min (c) 5–10 min (d) 15 min
8. Adult me rectal thermometer kitna andar?
   (a) 0.5 inch (b) 1.5 inch (3.5 cm) (c) 3 inch (d) 4 inch
9. Fan se garmi kam hona kaunsa mechanism?
   (a) Conduction (b) Radiation (c) Evaporation (d) Convection
10. Ice pack se heat loss –
    (a) Convection (b) Radiation (c) Evaporation (d) Conduction
11. Malaria me fever ka type –
    (a) Remittent (b) Relapsing (c) Intermittent (d) Continuous
12. Typhoid ke pehle hafte me fever pattern –
    (a) Quartan (b) Intermittent (c) Relapsing (d) Step-ladder
13. Normal adult pulse –
    (a) 40–60 (b) 60–100 (c) 100–140 (d) 120–160
14. Apical pulse kahan sunte hain (adult)?
    (a) Left 5th ICS mid-clavicular line (b) Left 2nd ICS (c) Xiphoid (d) Right 2nd ICS
15. Pulse deficit milta hai –
    (a) Fever (b) Atrial fibrillation (c) Anaemia (d) Shock
16. Normal respiration rate (adult) –
    (a) 8–10 (b) 12–20 (c) 24–30 (d) 30–40
17. Kussmaul breathing kis condition me?
    (a) Asthma (b) Opioid overdose (c) Diabetic ketoacidosis (d) Pneumothorax
18. Cheyne-Stokes breathing –
    (a) Snoring (b) Waxing-waning depth with apnoea periods (c) Deep rapid regular (d) Whistling

19. 1st Korotkoff sound –
(a) Systolic (b) Pulse pressure (c) Diastolic (d) Mean

20. Normal pulse pressure –
(a) 10 (b) 40 (c) 80 (d) 120

? Level 2 – Priority & Application

21. Patient ne abhi garam chai pi hai. Oral temperature kab lein?
(a) Axillary hi lo (b) Turant (c) 15–30 min baad (d) 2 hrs baad

22. MI patient ka temperature lena hai. Kaunsa route avoid karein?
(a) Tympanic (b) Rectal (c) Oral (d) Axillary

23. Patient ka pulse irregular hai. Kitni der count karein?
(a) 15 sec × 4 (b) 30 sec × 2 (c) 10 sec × 6 (d) Full 1 minute

24. Digoxin se pehle apical pulse 54/min hai (adult). Nurse –
(a) Dawa hold karke doctor ko inform kare (b) 1 hr baad de (c) Dose double kare (d) Dawa de de

25. Respiration count karte waqt nurse patient ko –
(a) Saans rokne ko kehti hai (b) Nahi batati, pulse lene ka bahana rakhti hai (c) Bata deti hai ki saans gin rahi hai (d) Tez saans lene ko kehti hai

26. Fever ke chill stage me nurse –
(a) Fan chalaye (b) Blanket se dhake, garam rakhe (c) Cold sponge kare (d) Ice pack lagaye

27. Mercury thermometer toot gaya. Sahi action –
(a) Pani se baha do (b) Haath se utha lo (c) Vacuum cleaner se saaf karo (d) Gloves pehen ke spill kit se collect karo, airtight container me rakho

28. BP lete waqt patient ka arm heart level se neeche hai. Reading –
(a) Falsely low (b) Normal (c) Falsely high (d) Zero

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

29. BP falsely HIGH kab NAHI aata?
(a) Cuff too wide (b) Cuff loose (c) Arm below heart (d) Cuff too narrow

30. Rectal temperature kis me contraindicated NAHI hai?
(a) Rectal surgery (b) Diarrhoea (c) Unconscious adult (d) MI patient

31. 102.2 °F = ? °C
(a) 38 °C (b) 39 °C (c) 40 °C (d) 41 °C

32. 40 °C = ? °F
(a) 100.4 °F (b) 102 °F (c) 104 °F (d) 106 °F

33. Match: P. Kussmaul Q. Cheyne-Stokes R. Stridor S. Wheeze — with 1. Asthma 2. Upper airway obstruction 3. DKA 4. Dying patient
(a) P-1, Q-4, R-2, S-3 (b) P-3, Q-2, R-4, S-1 (c) P-4, Q-3, R-1, S-2 (d) P-3, Q-4, R-2, S-1

34. Assertion (A): Pulse lene ke liye thumb use nahi karna chahiye. Reason (R): Thumb ki apni strong pulse hoti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

35. BP 130/70 ka MAP –
(a) 80 (b) 90 (c) 100 (d) 110

36. Pulse oximeter falsely high reading kab deta hai?
(a) Movement (b) Cold fingers (c) Carbon monoxide poisoning (d) Nail polish

37. Water-hammer (Corrigan's) pulse kis me milta hai?
(a) Aortic regurgitation (b) Heart block (c) Aortic stenosis (d) Shock

38. Pulsus paradoxus milta hai –
(a) Cardiac tamponade (b) Fever (c) Hypothyroidism (d) Anaemia

Answer Key & Explanation

1-c Hypothalamus body ka thermostat hai.
2-b 37 °C = 98.6 °F.
3-c > 41 °C (105.8 °F).
4-a < 35 °C (95 °F).
5-c Rectal core ke sabse kareeb.
6-b Axillary safe, lekin kam accurate.
7-c Axillary 5–10 (9) min.
8-b Adult 1.5 inch, child 1 inch, infant 0.5 inch.
9-d Hawa ka current = convection.
10-d Direct contact = conduction.
11-c Roz normal par aata hai = intermittent.
12-d Typhoid 1st week = step-ladder.
13-b 60–100 per min.
14-a Left 5th ICS MCL – apex beat.
15-b AF me apical > radial.
16-b 12–20 per min.
17-c Metabolic acidosis compensation.
18-b Dying/ HF/ ↑ICP.
19-a Phase I = systolic, phase V = diastolic.
20-b SBP − DBP ≈ 40.
21-c 15–30 min wait.
22-b Rectal se vagal stimulation → bradycardia.
23-d Irregular pulse full 1 minute.
24-a Adult < 60 hold karo.
25-b Bataane se breathing pattern badal jata hai.
26-b Chill stage me warmth do.
27-d Mercury toxic – spill kit, gloves.
28-c Arm neeche = falsely high.
29-a Wide cuff se falsely LOW.
30-c Unconscious adult me rectal allowed (oral nahi).
31-b (102.2 − 32) × 5/9 = 39 °C.
32-c 40 × 9/5 + 32 = 104 °F.
33-d Kussmaul DKA, Cheyne-Stokes dying, stridor upper airway, wheeze asthma.
34-a Dono sahi, R reason hai.
35-b (130 + 2×70) ÷ 3 = 90.
36-c CO-Hb ko oximeter O_2-Hb padhta hai.
37-a Aortic regurgitation.
38-a Inspiration par SBP > 10 mmHg girna.

6 Health Assessment & Physical Examination

Level 1 – Direct

1. Physical examination ka first technique –
(a) Auscultation (b) Inspection (c) Percussion (d) Palpation

2. Abdomen examination ka sahi order –
(a) IPPA (b) Auscultation first (c) PIPA (d) Inspection, Auscultation, Percussion, Palpation

3. Temperature palpate karne ke liye hand ka part –
(a) Ulnar surface (b) Fingertips (c) Palm (d) Dorsum (back) of hand

4. Vibration (fremitus) feel karne ke liye –
(a) Fingertips (b) Dorsum (c) Thumb (d) Palm/ ulnar surface

5. Normal lung par percussion note –
(a) Resonant (b) Flat (c) Dull (d) Tympanic

6. Liver par percussion note –
(a) Resonant (b) Dull (c) Tympanic (d) Hyper-resonant

7. Emphysema me percussion –
(a) Tympanic (b) Flat (c) Dull (d) Hyper-resonant

8. Stethoscope ka bell kis liye?
(a) Breath sounds (b) Bowel sounds (c) Low-pitched sounds (murmur, S3) (d) High-pitched sounds

9. Normal bowel sounds –
(a) 5–30/min (b) 1–2/min (c) Absent (d) 50–60/min

10. Retina dekhne ka instrument –
(a) Ophthalmoscope (b) Proctoscope (c) Otoscope (d) Laryngoscope

11. Visual acuity (Snellen chart) ki doori –
(a) 10 ft (b) 20 ft (6 m) (c) 30 ft (d) 5 ft

12. Normal capillary refill –
(a) < 2–3 sec (b) 5 sec (c) 10 sec (d) 1 min

13. BMI ka formula –
(a) Weight ÷ height (cm) (b) Height ÷ weight (c) Weight × height (d) Weight (kg) ÷ height $(m)^2$

14. Asian standard me BMI 24 –
(a) Overweight (b) Underweight (c) Obese (d) Normal

15. Normal pupil size –
(a) 0.5–1 mm (b) 2–6 mm (c) 8–10 mm (d) 12 mm

Level 2 – Priority & Application

16. Elderly patient me skin turgor kahan check karein?
(a) Abdomen (b) Leg (c) Sternum/ clavicle (d) Hand ka back

17. Pap smear ke liye position –
(a) Sims' (b) Fowler's (c) Prone (d) Lithotomy

18. Rectal examination ke liye position –
(a) Trendelenburg (b) Supine (c) Sims' / knee-chest (d) Fowler's

19. Abdominal examination ke time patient ko –
(a) Standing (b) Prone (c) Supine, knees thoda flexed (d) Trendelenburg

20. Patient ke abdomen me pain wala area kab palpate karein?
(a) Beech me (b) Kabhi nahi (c) Sabse last me (d) Sabse pehle

21. Bowel sounds "absent" kehne se pehle kitni der sunein?
(a) 30 sec (b) 1 min (c) 3–5 min (d) 10 min

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

22. Match: P. Dull Q. Tympanic R. Flat S. Hyper-resonant — with 1. Stomach 2. Liver 3. Emphysema 4. Thigh muscle
(a) P-2, Q-1, R-4, S-3 (b) P-3, Q-1, R-4, S-2 (c) P-1, Q-2, R-3, S-4 (d) P-2, Q-4, R-1, S-3

23. Muscle power against gravity, but resistance nahi – grade?
(a) 1 (b) 2 (c) 3 (d) 4

24. Deep tendon reflex 2+ ka matlab –
(a) Hyperactive with clonus (b) Diminished (c) Normal (d) Absent

25. Assertion (A): Abdomen me auscultation palpation se pehle karte hain. Reason (R): Palpation bowel sounds ko badal sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

26. Tactile fremitus kis condition me BADHTA hai?
(a) Pleural effusion (b) Pneumonia (consolidation) (c) Emphysema (d) Pneumothorax

27. Clubbing ka angle –
(a) 160° (b) 180° se kam (c) > 180° (d) 90°

28. Mitral area (apex) kahan hai?
(a) Left 3rd ICS (b) Left 2nd ICS (c) Right 2nd ICS (d) Left 5th ICS MCL

29. Babinski positive (adult) kya darshata hai?
(a) Normal (b) Hypocalcaemia (c) Upper motor neuron lesion (d) Muscle disease

30. Pitting oedema 3+ me depth –
(a) 2 mm (b) 4 mm (c) 6 mm (d) 8 mm

Answer Key & Explanation

1-b Inspection pehla technique hai.
2-d Abdomen: I-A-P-P.
3-d Dorsum temperature ke liye sensitive hai.
4-d Palm/ ulnar surface – vibration.
5-a Lung resonant.
6-b Solid organ = dull.
7-d Air trapping = hyper-resonant.
8-c Bell = low-pitched; diaphragm = high.
9-a 5–30 per min.
10-a Ophthalmoscope = fundus/ retina.
11-b 20 ft (6 m).
12-a < 2–3 sec normal.
13-d Quetelet index.
14-a Asian: 23–24.9 overweight.
15-b 2–6 mm.
16-c Elderly ki hand skin loose hoti hai.

17-d Lithotomy – vaginal exam.
18-c Sims'/ knee-chest.
19-c Knees flexed = abdominal muscles relax.
20-c Painful area last.
21-c 3–5 min (kai texts 5 min).
22-a Dull liver, tympanic stomach, flat muscle, hyper-resonant emphysema.
23-c Grade 3.
24-c 2+ normal.
25-a Dono sahi, R reason hai.
26-b Solid medium vibration badhata hai.
27-c > 180° = clubbing.
28-d Apex = 5th ICS left MCL.
29-c UMN lesion (adult me abnormal).
30-c 3+ = 6 mm.

7 Infection Control, Asepsis, Sterilization & BMW

Level 1 – Direct

1. Cross infection rokne ka sabse effective tareeka –
(a) Hand hygiene (b) Antibiotics (c) Mask (d) Isolation

2. Hand washing ka concept kisne diya?
(a) Semmelweis (b) Lister (c) Pasteur (d) Koch

3. Most common hospital acquired infection –
(a) Bloodstream (b) Pneumonia (c) UTI (d) Wound infection

4. Hospital acquired infection admission ke kitne ghante baad?
(a) 12 hrs (b) 24 hrs (c) ≥ 48 hrs (d) 7 days

5. Alcohol hand rub ka time –
(a) 5 sec (b) 20–30 sec (c) 2 min (d) 5 min

6. Soap-water hand wash ka time –
(a) 10 sec (b) 40–60 sec (c) 5 min (d) 10 min

7. Autoclave ka standard –
(a) 100 °C, 30 min (b) 121 °C, 15 psi, 15–20 min (c) 160 °C, 2 hrs (d) 70 °C, 1 hr

8. Hot air oven –
(a) 121 °C, 15 min (b) 160 °C, 2 hrs (c) 100 °C, 10 min (d) 60 °C, 30 min

9. Endoscopes ke liye disinfectant –
(a) Alcohol 70 % (b) Bleaching powder (c) Savlon (d) 2 % Glutaraldehyde

10. Blood spill ke liye –
(a) Dettol (b) Water (c) 70 % alcohol (d) 1 % sodium hypochlorite

11. Disposable syringes ka sterilization –
(a) Gamma radiation (b) Autoclave (c) Hot air oven (d) Boiling

12. Heat-sensitive plastic items ke liye –
(a) Ethylene oxide (b) Flaming (c) Hot air oven (d) Boiling

13. Autoclave ka biological indicator –
(a) Geobacillus stearothermophilus (b) E. coli (c) Staph aureus (d) Bacillus atrophaeus

14. Sharps ke liye container –
(a) Yellow bag (b) Blue box (c) Red bag (d) White translucent puncture-proof

15. Glassware kis colour me?
(a) White (b) Red (c) Blue (d) Yellow

16. Contaminated plastic (IV set, catheter) –
(a) Black (b) White (c) Red (d) Yellow

17. BMW Rules kis saal ke hain (current)?
(a) 1998 (b) 2016 (c) 2020 (d) 2010

18. N95 mask kitne % particles filter karta hai?
(a) 50 % (b) 75 % (c) 95 % (d) 100 %

? Level 2 – Priority & Application

19. Nurse ko used needle se needle-stick injury hui. Pehla kadam –
(a) Khoon nichodo (b) Bleach lagayein (c) Soap & running water se dhoyein (d) Report kal karein

20. TB patient ke kamre me jaate waqt nurse pehnegi –
(a) N95 respirator (b) Sirf gloves (c) Kuch nahi (d) Surgical mask

21. MRSA wound infection wale patient ke liye precaution –
(a) Protective (b) Airborne (c) Droplet (d) Contact

22. Influenza patient – precaution –
(a) Airborne (b) Standard only (c) Droplet (d) Contact

23. PPE pehnte waqt sabse last kya pehnein?
(a) Mask (b) Gown (c) Gloves (d) Goggles

24. PPE utaarte waqt sabse pehle –
(a) Gown (b) Mask (c) Gloves (d) Goggles

25. Sterile field par kaam karte waqt sterile object waist se neeche chala gaya. Ab wo –
(a) Thoda sterile (b) Sterile hai (c) Unsterile maana jayega (d) Wapas use karo

26. Sterile pack kholte waqt pehla flap –
(a) Left side (b) Apne se door (c) Right side (d) Apni taraf

27. Neutropenic patient ke liye isolation –
(a) Contact (b) Droplet (c) Protective (reverse) (d) Airborne

28. Hands visibly soiled hain. Kya use karein?
(a) Tissue (b) Soap & water (c) Gloves pehen lo (d) Alcohol rub

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

29. Airborne precautions kis disease me NAHI?
(a) Chickenpox (b) Mumps (c) Measles (d) Tuberculosis

30. Sterilization ka matlab –
(a) Sirf bacteria kill (b) Spores ko chhod ke sab kill (c) Sirf virus kill (d) Spores sahit sab microorganisms kill

31. Spaulding classification me "critical" item –
(a) BP cuff (b) Surgical instruments & urinary catheter (c) Bedpan (d) Stethoscope

32. Match: P. Autoclave Q. Hot air oven R. Ethylene oxide S. Gamma rays — with 1. Disposable syringes 2. Plastic/ rubber items 3. Glassware, oils, powders 4. Linen, instruments
(a) P-4, Q-2, R-3, S-1 (b) P-3, Q-4, R-1, S-2 (c) P-1, Q-3, R-2, S-4 (d) P-4, Q-3, R-2, S-1

33. Assertion (A): Boiling sterilization ka method nahi hai. Reason (R): Boiling spores ko nahi maarta.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

34. Needle-stick ke baad transmission risk sabse zyada –
(a) HIV (b) Hepatitis C (c) Hepatitis B (d) Equal

35. Sterile field ke border ka kitna hissa unsterile?
(a) 5 inch (b) Pura sterile (c) 1 inch (2.5 cm) (d) 1 cm

36. 70 % alcohol 100 % se zyada effective kyun?
(a) Jaldi sookhta hai (b) Paani protein coagulation me madad karta hai (penetration) (c) Kam smell (d) Sasta hai

37. Hand hygiene ke WHO 5 moments me kaunsa NAHI hai?
(a) After body fluid exposure (b) After touching patient surroundings (c) Before touching patient (d) Before entering hospital gate

38. Chlorinated plastic bags BMW me banned kyun?

(a) Jalane par dioxins bante hain (b) Colour nahi hota (c) Fat jaate hain (d) Mehenge

Answer Key & Explanation

1-a Hand hygiene single most important.
2-a Semmelweis, 1847.
3-c UTI (CAUTI) sabse common.
4-c ≥ 48 hrs baad.
5-b 20–30 sec.
6-b 40–60 sec.
7-b 121 °C, 15 psi, 15–20 min.
8-b 160 °C, 2 hrs.
9-d Cidex – high-level disinfection.
10-d 1 % hypochlorite.
11-a Industrial gamma irradiation.
12-a ETO – low temperature gas.
13-a Geobacillus spores – autoclave.
14-d White translucent, puncture-proof.
15-c Blue – glassware.
16-c Red – recyclable plastic.
17-b BMW Rules 2016.
18-c ≥ 95 % of 0.3 µm particles.
19-c Dhoyein, squeeze nahi, phir report & PEP.
20-a TB airborne – N95.
21-d Contact – gloves + gown.
22-c Droplet – surgical mask.
23-c Gloves last (gown cuff ke upar).
24-c Gloves sabse contaminated – pehle utaro.
25-c Waist se neeche = unsterile.
26-b Door wala flap pehle – reach over nahi karna padta.
27-c Protective isolation – patient ko bachana.
28-b Visibly soiled → soap & water.
29-b Mumps droplet hai.
30-d Spores sahit sab.
31-b Sterile tissue/ blood contact = critical.
32-d Autoclave linen/ instruments, oven glass/ oils, ETO plastic, gamma syringes.
33-a Dono sahi, R reason hai.
34-c HBV ~30 % > HCV ~1.8 % > HIV ~0.3 %.
35-c 1 inch border unsterile.
36-b Water penetration ke liye zaroori hai.
37-d Hospital gate WHO moment nahi.
38-a Dioxins toxic & carcinogenic.

8 Personal Hygiene, Bed Making & Comfort Devices

Level 1 – Direct

1. Bed bath ke paani ka temperature –
   (a) 30–35 °C (b) 43–46 °C (c) 55–60 °C (d) 20–25 °C
2. Eye ko kis direction me saaf karte hain?
   (a) Outer to inner canthus (b) Circular (c) Inner to outer canthus (d) Upar se neeche
3. Female perineal care ki direction –
   (a) Side to side (b) Front to back (c) Back to front (d) Circular
4. Back massage me long, smooth gliding stroke –
   (a) Tapotement (b) Effleurage (c) Petrissage (d) Friction
5. Kneading (muscle ko daba ke uthana) stroke –
   (a) Tapotement (b) Petrissage (c) Vibration (d) Effleurage
6. Discharge ke baad patient unit me kaunsa bed?
   (a) Open bed (b) Cardiac bed (c) Occupied bed (d) Closed bed
7. Post-operative bed me top linen –
   (a) Hata dete hain (b) Sir ki taraf (c) Pair ki taraf fold (d) Side me fan-fold
8. Bed cradle ka use –
   (a) Feeding (b) Patient ko jhulana (c) Linen ka weight body se door rakhna (d) Pillow rakhna
9. Footdrop rokne ke liye –
   (a) Sandbag under head (b) Bed cradle (c) Footboard (d) Air ring
10. Hip ka external rotation rokne ke liye –
    (a) Trochanter roll (b) Knee gatch (c) Hand roll (d) Footboard
11. Head lice ka first-line treatment –
    (a) Lindane (b) Kerosene (c) Permethrin 1 % (d) Dettol
12. Nits kya hain?
    (a) Lice ke ande (b) Adult lice (c) Fungus (d) Dandruff
13. Mitred corner ka angle –
    (a) 30° (b) 45° (c) 60° (d) 90°
14. Hot water bottle (adult) ka temperature –
    (a) 30–35 °C (b) 46–52 °C (c) 60–70 °C (d) 80 °C

Level 2 – Priority & Application

15. Unconscious patient ki oral care ke liye position –
    (a) Supine flat (b) High Fowler's (c) Prone (d) Side-lying (lateral) with head turned
16. Unconscious patient ka mouth kholne ke liye –
    (a) Padded tongue blade/ bite block (b) Metal spoon (c) Fingers (d) Forcefully

17. Dentures nikalte waqt pehle –
(a) Upper (b) Koi bhi (c) Dono saath (d) Lower

18. Dentures saaf karte waqt –
(a) Sink me towel/ paani rakh ke, gunguna paani (b) Garam paani (c) Bleach (d) Sukha brush

19. Diabetic patient ke toenails –
(a) Nurse round cut kare (b) Corners deep kaate (c) Blade se (d) Podiatrist se/ seedha file

20. Bed bath me paani kab badlein?
(a) Kabhi nahi (b) Sirf end me (c) Har 2 min (d) Thanda/ ganda hone par & perineal care se pehle

21. Linen ke saath kaunsa kaam GALAT hai?
(a) Wrinkle-free bottom sheet (b) Uniform se door rakhna (c) Ganda linen floor par na rakhna (d) Linen ko jhaadna (shake)

22. Male patient ki perineal care ke baad foreskin –
(a) Tape karo (b) Kaat do (c) Peeche hi rehne do (d) Wapas aage kar do

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

23. Back massage kis me contraindicated NAHI hai?
(a) DVT (b) Reddened bony prominence (c) Rib fracture (d) Bed-ridden patient with intact skin

24. Match: P. Cardiac bed Q. Fracture bed R. Amputation bed S. Post-op bed — with 1. Fracture boards 2. Backrest & cardiac table 3. Tourniquet at bedside 4. Top linen fan-folded to side
(a) P-1, Q-2, R-4, S-3 (b) P-2, Q-3, R-1, S-4 (c) P-2, Q-1, R-3, S-4 (d) P-3, Q-1, R-2, S-4

25. Assertion (A): Unconscious patient ki oral care side-lying position me karte hain. Reason (R): Isse fluid aspiration ka risk kam hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

26. Air ring/ donut cushion ab recommend kyun nahi?
(a) Infection (b) Bhaari hai (c) Mehenga (d) Centre me circulation kam karke ulcer badhata hai

27. Partial bed bath me kaun se parts saaf hote hain?
(a) Face, hands, axilla, back, perineum (b) Sirf sir (c) Sirf pair (d) Pura body

28. Cheilosis (lip corners cracking) kis vitamin ki kami?
(a) D (b) C (c) B1 (d) B2 (riboflavin)

✓ Answer Key & Explanation

1-b 43–46 °C (110–115 °F).
2-c Inner → outer (nasolacrimal duct se door).
3-b Front to back – urethra me infection nahi.
4-b Effleurage – gliding.
5-b Petrissage – kneading.
6-d Closed bed – naye patient ka intezaar.
7-d Side me fan-fold – patient ko aasani se shift karne ke liye.
8-c Burns, fracture, gout me linen ka weight hatata hai.
9-c Footboard dorsiflexion maintain karta hai.
10-a Trochanter roll.
11-c Permethrin 1 %.
12-a Nits = eggs (hair shaft se chipke).
13-b 45°.
14-b Adult 46–52 °C.
15-d Aspiration prevention.
16-a Fingers kabhi nahi (bite).
17-a Upper denture pehle.
18-a Garam paani denture ko tedha kar deta hai.
19-d Injury/ infection risk – podiatrist.
20-d Thanda/ soapy/ dirty + perineal se pehle.
21-d Shake karne se microbes failte hain.
22-d Paraphimosis rokne ke liye.
23-d Intact skin par massage theek hai.
24-c Cardiac backrest, fracture boards, amputation tourniquet, post-op fan-fold.

25-a Dono sahi, R reason.
26-d Pressure centre par → ischaemia.
27-a Odour/ discomfort wale parts.
28-d Riboflavin deficiency.

9 Rest, Sleep & Pain

Level 1 – Direct

1. Sleep-wake cycle ko control karne wala hormone –
   (a) Melatonin (b) Insulin (c) Thyroxine (d) Cortisol
2. Dreaming kis stage me hota hai?
   (a) NREM 3 (b) REM (c) NREM 1 (d) NREM 2
3. Sabse deep sleep stage –
   (a) N1 (b) REM (c) N3 (delta) (d) N2
4. Growth hormone release kis stage me?
   (a) N3 (b) REM (c) N1 (d) Awake
5. Ek sleep cycle ka time –
   (a) 30 min (b) 90 min (c) 3 hrs (d) 5 hrs
6. Newborn ki sleep requirement –
   (a) 6–8 hrs (b) 14–17 hrs (c) 10 hrs (d) 20–24 hrs
7. Most common sleep disorder –
   (a) Somnambulism (b) Narcolepsy (c) Sleep apnoea (d) Insomnia
8. Sleepwalking kehlata hai –
   (a) Bruxism (b) Somniloquy (c) Enuresis (d) Somnambulism
9. Gate control theory kisne di?
   (a) Melzack & Wall (b) Freud (c) Maslow (d) Selye
10. "Pain is whatever the patient says it is" kisne kaha?
    (a) Henderson (b) Melzack (c) Nightingale (d) Margo McCaffery
11. 4 saal ke bachche ke liye pain scale –
    (a) NRS 0–10 (b) Wong-Baker FACES (c) VAS (d) PAINAD
12. Dementia patient ke liye pain scale –
    (a) PAINAD (b) CRIES (c) FLACC (d) FACES
13. Opioid antidote –
    (a) Naloxone (b) Flumazenil (c) Atropine (d) Vitamin K
14. Paracetamol ki max adult dose/day –
    (a) 1 g (b) 2 g (c) 4 g (d) 8 g
15. Amputated limb me dard –
    (a) Phantom (b) Radiating (c) Referred (d) Psychogenic

Level 2 – Priority & Application

16. Patient ko neend nahi aati. Pehla nursing measure –
    (a) Raat bhar TV (b) Sleeping pill (c) Sleep hygiene – routine, caffeine avoid, back rub, quiet room (d) Din me sone do
17. Patient morphine par hai, RR 9/min. Nurse –
    (a) Next dose de (b) Pani pilaye (c) Dose hold kare, doctor ko inform, naloxone ready (d) Ignore kare

18. Dressing se pehle analgesic kab dein?
(a) 5 min pehle (b) 30 min pehle (c) 2 hrs pehle (d) Dressing ke baad

19. PCA ka button kaun dabata hai?
(a) Nurse (b) Relatives (c) Sirf patient (d) Doctor

20. Opioid lene wale patient ko constipation rokne ke liye –
(a) Prophylactic laxative/ fibre, fluids (b) Kuch nahi (c) Opioid band (d) Antacid

21. Patient pain 8/10 batata hai par muskura raha hai. Nurse –
(a) Pain ko ignore kare (b) Patient ki report par vishwas karke manage kare (c) Placebo de (d) Psychiatrist bulaye

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

22. Acute pain ka sign kaunsa NAHI hai?
(a) Tachycardia (b) Sweating (c) Adaptation – vital signs normal (d) Dilated pupils

23. Match: P. MI Q. Gallbladder R. Diaphragm S. Kidney stone — referred pain to: 1. Right shoulder 2. Left arm/ jaw 3. Groin 4. Shoulder tip
(a) P-1, Q-2, R-3, S-4 (b) P-2, Q-1, R-4, S-3 (c) P-2, Q-4, R-1, S-3 (d) P-3, Q-1, R-4, S-2

24. Assertion (A): Massage aur TENS pain kam karte hain. Reason (R): Large fibres ka stimulation "gate" band karta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

25. Opioid ka kaunsa side effect tolerance develop nahi karta?
(a) Sedation (b) Nausea (c) Respiratory depression (d) Constipation

26. WHO analgesic ladder step 2 me –
(a) Codeine/ tramadol (b) Morphine (c) Only paracetamol (d) Fentanyl

27. Night terrors kis stage me hote hain?
(a) Awake (b) REM (c) N3 (d) N1

28. Warm milk neend kyun laata hai?
(a) Sugar (b) Tryptophan (c) Calcium (d) Fat

✓ Answer Key & Explanation

1-a Pineal gland – melatonin.
2-b REM = dreaming.
3-c N3 = slow-wave/ delta.
4-a Deep sleep me GH.
5-b ~90 min, raat me 4–6 cycles.
6-b 14–17 hrs.
7-d Insomnia.
8-d Somnambulism – N3 me.
9-a Melzack & Wall, 1965.
10-d McCaffery, 1968.
11-b FACES ≥ 3 yrs.
12-a PAINAD – advanced dementia.
13-a Naloxone.
14-c 4 g/day (hepatotoxicity).
15-a Phantom pain real hota hai.
16-c Non-pharmacological pehle.
17-c RR < 12 = respiratory depression.
18-b Oral 30–60 min, IV 15–30 min pehle.
19-c PCA by proxy dangerous.
20-a Constipation me tolerance nahi hota.
21-b Self-report most reliable.
22-c Chronic pain me adaptation hota hai.
23-b MI arm/ jaw, GB right shoulder, diaphragm shoulder tip, kidney groin.
24-a Gate control theory.
25-d Constipation persists.
26-a Weak opioid step 2.
27-c Night terrors N3, nightmares REM.
28-b Tryptophan → serotonin/ melatonin.

10 Body Mechanics, Mobility, Positions & Pressure Ulcers

Level 1 – Direct

1. Standing adult ka centre of gravity –
   (a) Knees (b) Chest (c) S2 vertebra (pelvis) (d) Umbilicus
2. Haath ki hatheli upar karna –
   (a) Pronation (b) Inversion (c) Supination (d) Abduction
3. Body ke midline se door le jana –
   (a) Flexion (b) Abduction (c) Adduction (d) Rotation
4. Footdrop ka matlab –
   (a) Abduction (b) Dorsiflexion contracture (c) Plantar flexion contracture (d) Eversion
5. Muscle contract ho par joint move na ho –
   (a) Aerobic (b) Isotonic (c) Isometric (d) Isokinetic
6. Passive ROM kya maintain karta hai?
   (a) Muscle strength (b) Muscle mass (c) Endurance (d) Joint mobility
7. Bed sore ka most common site –
   (a) Elbow (b) Occiput (c) Heel (d) Sacrum
8. Braden scale me kitne parameters?
   (a) 4 (b) 5 (c) 6 (d) 7
9. Braden score "at risk" –
   (a) ≥ 20 (b) ≤ 18 (c) 23 (d) ≥ 25
10. Stage 1 pressure ulcer –
    (a) Blister (b) Bone exposed (c) Non-blanchable redness, skin intact (d) Fat visible
11. Bone/ tendon exposed – stage?
    (a) 1 (b) 2 (c) 3 (d) 4
12. Bed-ridden patient ko kitni der me turn karein?
    (a) Every 30 min (b) Every 6 hrs (c) Once a day (d) Every 2 hrs
13. Sabse stable crutch gait –
    (a) 2-point (b) Swing-through (c) 4-point (d) 3-point
14. Crutch ki length – axilla se kitna neeche?
    (a) 2–3 finger (≈ 5 cm) (b) 15 cm (c) Touching axilla (d) 30 cm
15. Paraplegic patient ke liye gait –
    (a) 3-point (b) Swing-through (c) 4-point (d) 2-point

Level 2 – Priority & Application

16. Heavy object uthate waqt sahi tareeka –
    (a) Knees bend karke, object body ke paas, back straight (b) Twist karke (c) Haath seedhe karke (d) Kamar jhuka ke

17. Crutch par chalne wala patient seedhiyan CHADH raha hai. Pehle –
(a) Good (unaffected) leg (b) Crutches (c) Bad leg (d) Dono saath

18. Crutch se seedhiyan UTARTE waqt pehle –
(a) Sirf crutches (b) Good leg (c) Koi bhi (d) Crutches + affected leg

19. Cane kis haath me?
(a) Affected side (b) Dominant always (c) Unaffected (strong) side (d) Dono

20. Walker se chalte waqt kram –
(a) Dono legs, phir walker (b) Good leg, walker, bad leg (c) Walker drag karo (d) Walker aage, affected leg, phir good leg

21. Hemiplegic patient ko bed se wheelchair – wheelchair kahan rakhein?
(a) Bed ke peeche (b) Unaffected (strong) side, brakes locked (c) Door ke paas (d) Affected side

22. Spinal injury patient ko turn karne ka tareeka –
(a) Akele nurse (b) Pull sheet (c) Patient khud (d) Log rolling

23. Bed se uthne se pehle "dangling" kyun?
(a) Comfort (b) Orthostatic hypotension rokne ke liye (c) Exercise (d) Feeding

24. Patient ko bed par upar khiskaane ke liye –
(a) Pair se dhakka (b) Haath pakad ke kheencho (c) Lift/ draw sheet + 2 nurses, HOB flat (d) Drag karo

25. Crutch walker ki body weight kahan?
(a) Elbows (b) Axilla (c) Shoulders (d) Hands

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

26. Immobility ka complication kaunsa NAHI hai?
(a) Hypertrophy of muscles (b) DVT (c) Hypostatic pneumonia (d) Renal calculi

27. Pressure ulcer prevention me kaunsa GALAT hai?
(a) Turn every 2 hrs (b) HOB ≤ 30° (c) Reddened area par massage (d) High protein diet

28. Match: P. Sims' Q. Lithotomy R. Knee-chest S. Trendelenburg — with 1. Sigmoidoscopy 2. Enema 3. Pelvic surgery 4. Vaginal exam/ delivery
(a) P-2, Q-1, R-4, S-3 (b) P-2, Q-4, R-1, S-3 (c) P-1, Q-4, R-2, S-3 (d) P-4, Q-2, R-3, S-1

29. Assertion (A): HOB 30° se upar rakhne se sacral pressure ulcer ka risk badhta hai. Reason (R): Patient neeche slide karta hai jisse shear force paida hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Isometric exercise kis patient me avoid?
(a) Post-op leg (b) Cardiac patient (c) Elderly healthy (d) Cast patient

31. Pressure ulcer base slough/ eschar se dhaka hai –
(a) Unstageable (b) Stage 1 (c) Stage 2 (d) Stage 3

32. Healing stage 4 ulcer ko kya kehte hain?
(a) Healing stage 4 (b) Stage 3 (c) Stage 1 (d) Stage 2

33. Braden scale ka kaunsa parameter NAHI hai?
(a) Moisture (b) Age (c) Nutrition (d) Friction & shear

✓ Answer Key & Explanation

1-c S2 – pelvis ka centre.
2-c Supination = palm up.
3-b Abduction = away.
4-c Plantar flexion me fix.
5-c Isometric – no movement.
6-d Joint mobility; strength nahi.
7-d Sacrum/ coccyx.
8-c 6 subscales.
9-b ≤ 18 at risk.
10-c Non-blanchable erythema.
11-d Stage 4.
12-d Har 2 hrs.
13-c 4-point – 3 points of support.
14-a 2–3 finger widths.
15-b Swing-to/ swing-through.
16-a Leg muscles use karo, back nahi.
17-a "Up with the good".
18-d "Down with the bad".
19-c Strong side ke haath me.
20-d Walker → affected → unaffected.
21-b Strong side par pivot.
22-d Log roll – spine aligned.
23-b Postural hypotension se girna rokta hai.
24-c Drag se shear/ friction.
25-d Axilla par weight = crutch palsy.
26-a Immobility se atrophy hoti hai.
27-c Red area par massage nuksaan karti hai.
28-b Sims enema, lithotomy vaginal, knee-chest sigmoidoscopy, Trendelenburg pelvic.
29-a Dono sahi, R reason.
30-b Isometric se BP & HR badhte hain.
31-a Unstageable.
32-a Reverse staging nahi karte.
33-b Age Braden me nahi.

11 Nutrition & Feeding

Level 1 – Direct

1. 1 g fat se kitni calories?
(a) 4 (b) 7 (c) 9 (d) 12

2. 1 g protein se –
(a) 4 kcal (b) 9 kcal (c) 2 kcal (d) 7 kcal

3. Vitamin C ka sabse rich source –
(a) Amla (b) Orange (c) Tomato (d) Lemon

4. Night blindness kis vitamin ki kami?
(a) D (b) A (c) B1 (d) C

5. Pellagra ke 3 D's – kis vitamin ki kami?
(a) Niacin (b) Thiamine (c) Folic acid (d) Riboflavin

6. Scurvy –
(a) Vit D (b) Vit C (c) Vit A (d) Vit K

7. Beriberi –
(a) B1 (b) B12 (c) C (d) B6

8. Rickets –
(a) Vit E (b) Vit D (c) Vit K (d) Vit A

9. Oedema ke saath protein deficiency –
(a) Marasmus (b) Scurvy (c) Rickets (d) Kwashiorkor

10. Surgery ke baad pehli diet –
(a) Full liquid (b) Clear liquid (c) Regular (d) Soft

11. NG tube ki length measure karne ka method –
(a) Mouth-ear-sternum (b) Nose-chin-umbilicus (c) Ear-nose-navel (d) Nose-earlobe-xiphoid (NEX)

12. NG tube placement confirm karne ka gold standard –
(a) Patient bol sake (b) Air sound (whoosh test) (c) X-ray (d) Bubbles in water

13. Gastric aspirate ka pH –
(a) ≤ 5 (b) 7 (c) 8 (d) ≥ 9

14. NG feed ke baad flush –
(a) 100 ml (b) 30 ml water (c) 5 ml (d) Flush nahi

15. SAM child ka MUAC –
(a) < 11.5 cm (b) < 13.5 cm (c) < 15 cm (d) < 10 cm

Level 2 – Priority & Application

16. NG feed dete waqt patient ki position –
(a) Semi/ high Fowler's (30–45°+) (b) Prone (c) Supine (d) Trendelenburg

17. NG insertion ke time patient khaansne laga, cyanosis hua. Nurse –
(a) Paani pilaye (b) Tape kare (c) Tube turant withdraw kare (d) Aage badhaye

18. Gastric residual 600 ml hai. Nurse –
(a) Feed de de (b) Residual phenk de (c) Feed hold karke reassess & inform (d) Double feed

19. TPN bag khatam, naya nahi aaya. Nurse –
(a) Line band kar de (b) D10W same rate par lagaye (c) NS bolus (d) Kuch nahi

20. NG tube insert karte waqt lubricant –
(a) Vaseline (oil-based) (b) Coconut oil (c) Ghee (d) Water-soluble jelly

21. Visually impaired patient ko khana –
(a) Sirf liquid do (b) Plate hata do (c) Khud khila do (d) Clock method se food position batao

22. Stroke patient ko feed karne se pehle –
(a) Turant solid (b) Swallow screening (c) Straw se paani (d) Supine position

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

23. Clear liquid diet me kya NAHI aata?
(a) Apple juice (b) Clear soup (c) Milk (d) Jelly

24. Low-sodium diet kis me NAHI chahiye?
(a) Heart failure (b) Cirrhosis with ascites (c) Hypertension (d) Addison's disease

25. Match: P. Gout Q. Celiac disease R. Hepatic encephalopathy S. Constipation — with 1. Low protein 2. High fibre 3. Low purine 4. Gluten-free
(a) P-3, Q-1, R-4, S-2 (b) P-1, Q-4, R-3, S-2 (c) P-3, Q-4, R-1, S-2 (d) P-4, Q-3, R-2, S-1

26. Assertion (A): Enteral feeding parenteral se behtar hai jab gut kaam kar raha ho. Reason (R): Enteral feeding gut mucosa maintain karti hai aur infection kam karti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

27. Tube feeding ka most common complication –
(a) Sinusitis (b) Diarrhoea (c) Hyperglycaemia (d) Aspiration

28. Kaunsi tablet crush NAHI karni chahiye?
(a) Vitamin B complex plain (b) Enteric-coated aspirin (c) Paracetamol plain (d) Metformin IR

29. Fat-soluble vitamins –
(a) B, C (b) A, D, E, K (c) B12, C (d) B1, B2

30. Kwashiorkor ka feature NAHI hai –
(a) Moon face (b) Flag sign (c) Severe wasting without oedema ("old man face") (d) Oedema

✓ Answer Key & Explanation

1-c Fat 9 kcal/g.
2-a Protein 4 kcal/g.
3-a Amla richest.
4-b Vit A – first sign night blindness.
5-a Niacin (B3).
6-b Vit C – bleeding gums.
7-a Thiamine.
8-b Vit D.
9-d Kwashiorkor – oedema.
10-b Clear liquid pehle.
11-d NEX method.
12-c X-ray most accurate.
13-a pH ≤ 5 = gastric.
14-b 30 ml water.
15-a < 11.5 cm.
16-a Aspiration prevention.
17-c Trachea me hai – withdraw.
18-c High residual – hold & inform.
19-b Abrupt stop → rebound hypoglycaemia.
20-d Oil → lipid pneumonia.
21-d Clock method.
22-b Aspiration risk check pehle.

23-c Milk full liquid me aata hai.
24-d Addison's me sodium loss – high Na diet.
25-c Gout low purine, celiac gluten-free, HE low protein, constipation high fibre.
26-a Dono sahi, R reason.
27-b Diarrhoea most common, aspiration most serious.
28-b Enteric-coated crush nahi.
29-b A, D, E, K.
30-c Old man face marasmus ka feature hai.

12 Urinary Elimination & Catheterization

Level 1 – Direct

1. Normal adult urine output/day –
(a) 500 ml (b) 1–1.5 L (c) 3–4 L (d) 5 L

2. Oliguria –
(a) Painful urination (b) < 400 ml/day (c) > 3 L/day (d) < 100 ml/day

3. Anuria –
(a) < 1 L/day (b) < 400 ml/day (c) < 100 ml/day (d) Night urination

4. Normal urine specific gravity –
(a) 1.000 (b) 1.005–1.030 (c) 1.040–1.060 (d) 0.950

5. Painful urination –
(a) Dysuria (b) Polyuria (c) Nocturia (d) Enuresis

6. Normal bladder capacity –
(a) 50–100 ml (b) 300–600 ml (c) 1500 ml (d) 2000 ml

7. Female urethra ki length –
(a) 1 cm (b) 3–4 cm (c) 10 cm (d) 18–20 cm

8. Adult female ke liye catheter size –
(a) 8 Fr (b) 12–14 Fr (c) 20–22 Fr (d) 24 Fr

9. Foley balloon kis se fill karte hain?
(a) Air (b) Normal saline (c) Sterile water (d) Tap water

10. Female catheter kitna insert karte hain (urine aane tak)?
(a) 1–2 cm (b) 5–7.5 cm (c) 15 cm (d) 20 cm

11. Male catheter insertion length –
(a) 5 cm (b) 10 cm (c) 17–22.5 cm (d) 30 cm

12. Female catheterization ki position –
(a) Trendelenburg (b) Prone (c) Dorsal recumbent (d) Sims'

13. Urine sugar ke liye Benedict's test me "+ + + +" ka rang –
(a) Blue (b) Green (c) Yellow (d) Brick red

14. Stress incontinence ke liye exercise –
(a) Push-ups (b) ROM (c) Kegel (d) Isometric quadriceps

Level 2 – Priority & Application

15. 24-hr urine collection shuru karte waqt –
(a) Pehla subah ka urine discard karo, time note karo (b) Pehla subah ka urine collect karo (c) Raat ka urine (d) Random sample

16. 24-hr collection ke beech ek sample galti se phenk diya gaya. Nurse –
(a) Collection dobara shuru kare (b) Estimate likhe (c) Continue kare (d) Ignore

17. Catheterized patient se culture sample –
(a) Bag se (b) Sampling port se sterile syringe (c) Catheter disconnect karke (d) Meatus se

18. Catheter galti se vagina me chala gaya. Nurse –
(a) Doctor ko bulaye (b) Balloon inflate kare (c) Use wahi chhod ke naya sterile catheter meatus me daale (d) Nikal ke wahi dobara daale

19. Urine bag kahan hona chahiye?
(a) Bed par (b) Pet par (c) Bladder level se neeche, floor se upar (d) Side rail par

20. Post-op patient 8 hrs se void nahi kar paya. Pehla nursing action –
(a) Diuretic (b) Privacy, running water sound, warm water on perineum, normal position (c) Turant catheter (d) Fluid restrict

21. Catheter insert karte waqt balloon kab inflate karein?
(a) Insert karte hi (b) Pehle hi (c) Urine flow ke baad aur thoda aage badha ke (d) Kabhi nahi

22. Catheter remove karne ke baad patient ko first void kitni der me?
(a) 1 hr (b) 6–8 hrs (c) 24 hrs (d) 48 hrs

23. Male catheterization ke baad foreskin ko –
(a) Tape (b) Peeche (c) Kuch nahi (d) Wapas aage (replace)

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

24. CAUTI prevention me kaunsa GALAT hai?
(a) Remove ASAP (b) Routine daily catheter change (c) Bag below bladder (d) Closed drainage

25. Match: P. Stress Q. Urge R. Overflow S. Functional — incontinence with: 1. BPH dribbling 2. Coughing par leak 3. Dementia – toilet tak nahi pahunch pata 4. Sudden strong urge
(a) P-1, Q-4, R-2, S-3 (b) P-2, Q-1, R-4, S-3 (c) P-4, Q-2, R-3, S-1 (d) P-2, Q-4, R-1, S-3

26. Assertion (A): Foley balloon me saline nahi bharni chahiye. Reason (R): Saline crystallize hokar balloon deflate nahi hone deti.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

27. Urine ka fruity smell kis condition me?
(a) UTI (b) Dehydration (c) Liver failure (d) Diabetic ketoacidosis

28. CBI (TURP ke baad) me total output 3000 ml, irrigation 2200 ml. True urine output –
(a) 3000 ml (b) 800 ml (c) 5200 ml (d) 2200 ml

29. Retention aur suppression me farak –
(a) Retention – urine banta hai par void nahi; suppression – kidney urine nahi banati (b) Same hai (c) Retention – kidney fail (d) Suppression – bladder full

30. Male patient me catheter tape kahan karein?
(a) Kahin nahi (b) Lower abdomen/ upper thigh (penis upward) (c) Scrotum (d) Inner thigh

✓ Answer Key & Explanation

1-b ~1500 ml/day.
2-b < 400 ml/day.
3-c < 100 ml/day.
4-b 1.005–1.030.
5-a Dysuria.
6-b 300–600 ml.
7-b 3–4 cm – isliye UTI zyada.
8-b 12–14 Fr.
9-c Sterile water.
10-b 5–7.5 cm + 2.5 cm aur.
11-c 17–22.5 cm (7–9 inch).
12-c Dorsal recumbent.
13-d Brick red = max sugar.
14-c Kegel – pelvic floor.
15-a Pehla void discard, time note.
16-a Incomplete collection invalid.

17-b Port + sterile syringe.
18-c Galat catheter landmark ka kaam karta hai.
19-c Backflow & contamination rokne ke liye.
20-b Non-invasive pehle.
21-c Urethra me inflate karne se injury.
22-b 6–8 hrs.
23-d Paraphimosis prevention.
24-b Routine change ki zaroorat nahi.
25-d Stress cough, urge sudden, overflow BPH, functional dementia.
26-a Dono sahi, R reason.
27-d Ketones – fruity/ acetone.
28-b 3000 − 2200 = 800 ml.
29-a Retention vs suppression.
30-b Peno-scrotal pressure se bachne ke liye.

13 Bowel Elimination – Enema & Suppository

Level 1 – Direct

1. Enema dene ki position –
   (a) Left lateral (Sims') (b) Fowler's (c) Prone (d) Right lateral
2. Adult me rectal tube kitni andar?
   (a) 2.5 cm (b) 7.5–10 cm (3–4 inch) (c) 15 cm (d) 20 cm
3. Enema can ki height (adult) –
   (a) 10 cm (b) 30–45 cm (12–18 inch) (c) 90 cm (d) 1 m
4. Enema solution ka temperature (adult) –
   (a) 30 °C (b) 40.5–43 °C (c) 50 °C (d) 60 °C
5. Soap suds enema me soap ki quantity –
   (a) 5 ml in 1000 ml (b) 50 ml (c) 100 ml (d) 1 tablespoon in 100 ml
6. Oil retention enema ka purpose –
   (a) Gas nikalna (b) Diagnosis (c) Hyperkalaemia (d) Hard stool ko soft karna
7. Hypertonic (Fleet/ phosphate) enema ki volume –
   (a) 120 ml (b) 500 ml (c) 1000 ml (d) 2000 ml
8. Adult cleansing enema volume –
   (a) 100 ml (b) 750–1000 ml (c) 2000 ml (d) 50 ml
9. Infant ki rectal tube insertion –
   (a) 2.5–3.75 cm (b) 7.5 cm (c) 10 cm (d) 15 cm
10. Flatus tube max kitni der?
    (a) 5 min (b) 20–30 min (c) 2 hrs (d) 6 hrs
11. Melena (black tarry stool) –
    (a) Lower GI bleed (b) Obstructive jaundice (c) Upper GI bleed (d) Cholera
12. Clay-coloured stool –
    (a) Upper GI bleed (b) Iron therapy (c) Normal (d) Obstructive jaundice
13. Bulk-forming laxative –
    (a) Isabgol (psyllium) (b) Lactulose (c) Bisacodyl (d) Liquid paraffin
14. MGW enema me "M" –
    (a) Mineral oil (b) Methyl (c) Milk (d) Magnesium sulphate

Level 2 – Priority & Application

15. Enema ke dauran patient ko cramps hue. Nurse –
    (a) Tube nikal ke phenk de (b) Fast flow kare (c) Flow rok de/ can neeche kare, deep breathing karwaye (d) Ignore kare
16. Rectal tube insert karte waqt patient ko kehna –
    (a) Deep breath lo/ bear down karo (b) Saans rok lo (c) Khaanso (d) Chupchaap raho
17. Faecal impaction ka sign –
    (a) Rice-water stool (b) No stool, distension, liquid stool ooze (c) Clay stool (d) Bright red blood

18. Digital removal ke dauran patient ka pulse gir gaya. Nurse –
(a) Procedure rok de (vagal stimulation) (b) Continue (c) Enema de (d) Tez kare

19. Suppository kahan insert karte hain?
(a) Vagina me (b) Sirf anus par (c) Rectal wall ke saath, internal sphincter ke beyond (d) Stool ke beech me

20. Constipation ka pehla nursing measure –
(a) Digital removal (b) Turant enema (c) Laxative daily (d) Fluids, high-fibre diet, exercise, regular toilet time

21. Stool for ova & parasites –
(a) Urine ke saath (b) Fridge me 24 hrs (c) Toilet paper ke saath (d) Warm & fresh, 30 min me lab

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

22. Enema kis condition me contraindicated NAHI hai?
(a) Recent rectal surgery (b) Intestinal obstruction (c) Acute appendicitis (d) Pre-operative bowel prep

23. Match: P. Soap suds Q. Oil retention R. Carminative S. Kayexalate — with 1. Hyperkalaemia 2. Flatus 3. Soften stool 4. Cleansing
(a) P-3, Q-4, R-1, S-2 (b) P-4, Q-3, R-2, S-1 (c) P-4, Q-2, R-3, S-1 (d) P-1, Q-3, R-2, S-4

24. Assertion (A): Enema left lateral position me dena chahiye. Reason (R): Sigmoid & descending colon left side me hain, gravity se flow aasaan.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

25. Valsalva manoeuvre kis patient me khatarnaak?
(a) Young healthy (b) Child (c) Cardiac & ↑ICP patient (d) Pregnant 1st trimester

26. "Enema till clear" me max kitne enemas?
(a) 1 (b) 3 (c) 6 (d) 10

27. Tap water enema baar-baar dena (infant) se khatra –
(a) Water intoxication/ electrolyte imbalance (b) Hypoglycaemia (c) Hypernatraemia (d) Kuch nahi

28. Occult blood test se 3 din pehle kya avoid?
(a) Red meat, iron, Vit C, NSAIDs (b) Milk (c) Bread (d) Rice

✓ Answer Key & Explanation

1-a Left lateral (Sims').
2-b 7.5–10 cm.
3-b 30–45 cm.
4-b 40.5–43 °C.
5-a Castile soap 5 ml/ L.
6-d Hard stool soften – 30 min–3 hrs retain.
7-a 120 ml.
8-b 750–1000 ml.
9-a 2.5–3.75 cm (1–1.5 inch).
10-b 20–30 min se zyada sphincter damage.
11-c Upper GI bleed.
12-d Bile intestine me nahi pahunchta.
13-a Isabgol – safest.
14-d Magnesium sulphate–Glycerine–Water.
15-c Flow rok do, can neeche karo.
16-a Bear down se external sphincter relax hota hai.
17-b Overflow liquid stool.
18-a Vagal stimulation – bradycardia.
19-c Faecal mass me absorb nahi hogi.
20-d Non-pharmacological pehle.
21-d Parasites ke liye fresh & warm.
22-d Pre-op prep me diya jata hai.
23-b Soap cleansing, oil soften, carminative flatus, Kayexalate K^+.
24-a Dono sahi, R reason.
25-c Bradycardia & ↑ICP.
26-b Max 3.
27-a Hypotonic water → water intoxication.
28-a False results se bachne ke liye.

14 Oxygenation & Respiratory Care

Level 1 – Direct

1. Hypoxia ka earliest sign –
(a) Cyanosis (b) Bradycardia (c) Coma (d) Restlessness/ confusion

2. Cyanosis tab dikhta hai jab reduced Hb –
(a) > 1 g/dL (b) > 5 g/dL (c) > 10 g/dL (d) > 15 g/dL

3. Oxygen cylinder ka colour (India) –
(a) Blue (b) Grey (c) Green (d) Black body, white shoulder

4. N_2O cylinder ka colour –
(a) Black (b) Blue (c) Brown (d) Orange

5. Nasal cannula ka flow rate –
(a) 1–6 L/min (b) 8–10 L/min (c) 10–15 L/min (d) 20 L/min

6. Simple face mask ka minimum flow –
(a) 1 L (b) 3 L (c) 5 L (d) 10 L

7. Sabse zyada O_2 concentration bina intubation –
(a) Venturi (b) Nasal cannula (c) Simple mask (d) Non-rebreather mask

8. COPD ke liye exact FiO_2 dene wala device –
(a) Simple mask (b) Face tent (c) Non-rebreather (d) Venturi mask

9. Humidifier me paani –
(a) Tap water (b) Mineral water (c) Sterile/ distilled water (d) Saline

10. Adult suction pressure –
(a) 50 mmHg (b) 100–150 mmHg (c) 200–250 mmHg (d) 300 mmHg

11. Ek baar suction max kitni der?
(a) 5 sec (b) 10–15 sec (c) 30 sec (d) 1 min

12. Tracheostomy cuff pressure –
(a) 5–10 mmHg (b) 20–25 mmHg (c) 40–50 mmHg (d) 60 mmHg

13. Nebulizer particle size jo lower airway tak pahunche –
(a) 1–5 µm (b) 10–20 µm (c) 50 µm (d) 100 µm

14. Steam inhalation ki doori –
(a) 5 cm (b) 23–30 cm (9–12 inch) (c) 60 cm (d) 1 m

Level 2 – Priority & Application

15. COPD patient SpO_2 86 %. O_2 shuru karna – target SpO_2 –
(a) 100 % (b) 88–92 % (c) 75 % (d) 95–100 %

16. Suction kab apply karein?
(a) Pura time (b) Insert karte waqt (c) Kabhi nahi (d) Withdraw karte waqt, intermittent, rotate

17. Suction se pehle aur baad –
(a) Patient ko lita do (b) Paani pilao (c) Kuch nahi (d) 100 % O_2 se hyperoxygenate

18. Trachea aur mouth dono suction karne hain, ek hi catheter –
(a) Trachea pehle, phir mouth (b) Koi bhi order (c) Nose pehle (d) Mouth pehle

19. Fresh tracheostomy tube accidentally nikal gayi. Nurse –
(a) Doctor ka intezaar (b) Paani pilao (c) Help bulao, stoma ko tracheal dilator se khula rakho, O_2 do (d) Chhed band kar do

20. Incentive spirometer use – sahi instruction –
(a) Tez saans bahar chhodo (b) Ek baar din me (c) Lait ke (d) Dheere aur gehri saans andar lo, 3–5 sec roko

21. MDI se bronchodilator & steroid dono lene hain. Order –
(a) Bronchodilator pehle, phir steroid (b) Koi bhi (c) Saath me (d) Steroid pehle

22. Postural drainage ka sahi time –
(a) Khane se pehle/ subah (b) Raat 12 baje (c) Khana khane ke turant baad (d) Kabhi bhi

23. O_2 therapy ke time patient ke relative bidi peena chahte hain. Nurse –
(a) Allow kare (b) Ignore (c) Window kholi (d) Mana kare – O_2 combustion support karti hai

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

24. Chest physiotherapy kis me contraindicated NAHI hai?
(a) Rib fracture (b) Pulmonary embolism (c) Haemoptysis (d) Bronchiectasis with thick secretions

25. Match: P. Nasal cannula Q. Simple mask R. Non-rebreather S. Venturi — FiO_2: 1. Precise 24–60 % 2. 24–44 % 3. 35–60 % 4. 80–95 %
(a) P-1, Q-3, R-4, S-2 (b) P-2, Q-4, R-3, S-1 (c) P-3, Q-2, R-1, S-4 (d) P-2, Q-3, R-4, S-1

26. Assertion (A): COPD me high-flow O_2 khatarnaak ho sakti hai. Reason (R): Inme breathing ka stimulus hypoxic drive hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

27. Suctioning ki most common complication –
(a) Diarrhoea (b) Hypertension (c) Hypoxia (d) Fever

28. Tracheostomy dressing me gauze –
(a) Normal gauze ko cut karke (b) Pre-cut/ keyhole drain gauze (c) Plastic (d) Cotton

29. Tracheostomy bedside kya zaroori hai?
(a) Glucometer (b) Bedpan (c) Thermometer (d) Obturator & spare tube

30. Pursed-lip breathing ka purpose –
(a) O_2 ↑ (b) Airway collapse rokna, CO_2 bahar (c) Sputum ↑ (d) Cough ↑

Answer Key & Explanation

1-d Restlessness earliest; cyanosis late.
2-b > 5 g/dL reduced Hb.
3-d Black body, white shoulder.
4-b N_2O – blue.
5-a 1–6 L/min.
6-c < 5 L par CO_2 rebreathing.
7-d NRBM ~80–95 %.
8-d Venturi – fixed FiO_2.
9-c Sterile/ distilled.
10-b 100–150 mmHg.
11-b 10–15 sec.
12-b 20–25 mmHg.
13-a 1–5 µm.
14-b 23–30 cm – burn se bachav.
15-b CO_2 retainers ke liye 88–92 %.
16-d Withdraw karte waqt.
17-d Hypoxia prevention.
18-a Clean (trachea) se dirty (mouth).
19-c Fresh tract band ho sakta hai – khula rakho.
20-d Slow deep inhalation + hold.

21-a Pehle airway kholo, phir steroid.
22-a Vomiting/ aspiration se bachav.
23-d O_2 combustion support karti hai.
24-d Bronchiectasis me CPT indicated hai.
25-d Cannula 24–44, mask 35–60, NRBM 80–95, Venturi precise.
26-a Dono sahi, R reason.
27-c Hypoxia most common.
28-b Cut gauze ke fibres aspirate ho sakte hain.
29-d Obturator & spare tube.
30-b Back pressure airways khule rakhta hai.

15 Fluids, IV Therapy & Blood Transfusion

Level 1 – Direct

1. Adult body me paani ka %–
   (a) 40 % (b) 60 % (c) 80 % (d) 90 %
2. ICF ka main cation –
   (a) K^+ (b) Cl^- (c) Ca^{2+} (d) Na^+
3. Fluid balance ka best indicator –
   (a) BP (b) Daily weight (c) Pulse (d) Thirst
4. Blood ke saath compatible fluid –
   (a) Ringer's lactate (b) D10W (c) D5W (d) 0.9 % NaCl
5. Hypotonic fluid –
   (a) D10W (b) 0.9 % NS (c) 0.45 % NaCl (d) 3 % NaCl
6. 18 G cannula ka colour –
   (a) Blue (b) Green (c) Yellow (d) Pink
7. 20 G cannula –
   (a) Orange (b) Green (c) Grey (d) Pink
8. Neonates ke liye cannula –
   (a) 14 G (b) 18 G (c) 24 G (yellow) (d) 16 G
9. Sabse bada (largest) cannula –
   (a) 24 G (b) 22 G (c) 18 G (d) 14 G
10. Micro-drip set ka drop factor –
    (a) 10 (b) 15 (c) 20 (d) 60
11. Blood ki storage temperature –
    (a) −20 °C (b) 2–6 °C (c) 20–24 °C (d) 37 °C
12. Platelets ki storage –
    (a) −30 °C (b) 2–6 °C (c) 20–24 °C with agitation (d) Freezer
13. Ek unit blood max kitni der me?
    (a) 1 hr (b) 4 hrs (c) 8 hrs (d) 12 hrs
14. Universal donor –
    (a) O −ve (b) AB +ve (c) A +ve (d) B −ve
15. ORS (WHO low osmolarity) ki osmolarity –
    (a) 245 mOsm/L (b) 311 mOsm/L (c) 150 (d) 400

Level 2 – Priority & Application

16. IV site par cool, pale, swollen skin aur flow slow – ye hai –
    (a) Septicaemia (b) Air embolism (c) Infiltration (d) Phlebitis
17. Infiltration me pehla kadam –
    (a) Infusion rok ke cannula nikalo, limb elevate karo (b) Flow badhao (c) Flush karo (d) Massage

18. IV site par redness, warmth, vein ke along tender cord –
(a) Phlebitis (b) Normal (c) Haematoma (d) Infiltration

19. Patient ko achanak dyspnoea, chest pain, hypotension IV line disconnect hone ke baad. Position –
(a) Left lateral + Trendelenburg (b) High Fowler's (c) Prone (d) Right side, head up

20. IV fluid tezi se chalne par crackles, JVD, dyspnoea. Nurse –
(a) Trendelenburg (b) Flow slow kare, high Fowler's, O_2, doctor ko inform (c) Fluid bolus (d) Flow badhaye

21. Transfusion shuru hone ke 10 min baad patient ko back pain, chills. Pehla kadam –
(a) Transfusion turant band kare (b) Speed kam kare (c) Doctor ko phone (d) Antihistamine de

22. Transfusion reaction ke baad IV line –
(a) Same blood set se NS flush (b) D5W chalao (c) Nikal do (d) Naye tubing se NS chalu rakho (KVO)

23. Blood transfusion ke pehle 15 min nurse –
(a) Relatives ko bole (b) Patient ke paas rahe, slow rate (c) Fast rate (d) Doosre ward me kaam

24. IV cannulation ke liye pehla site choice –
(a) Neck (b) Hand ki distal vein (c) Leg (d) Antecubital

? Level 3 – Advanced (NOT / Calculation / Match / Assertion-Reason)

25. 1000 ml NS 8 ghante me, drop factor 15. Drops/min –
(a) 21 (b) 31 (c) 42 (d) 62

26. 500 ml 4 ghante me, drop factor 20. Drops/min –
(a) 21 (b) 31 (c) 42 (d) 50

27. 1500 ml 12 ghante me – ml/hr –
(a) 100 (b) 125 (c) 150 (d) 200

28. Micro-drip (60), 50 ml/hr – microdrops/min –
(a) 25 (b) 50 (c) 60 (d) 100

29. 1000 ml at 25 drops/min, DF 15 – kitna time?
(a) 6 hrs (b) 8 hrs (c) 10 hrs (d) 12 hrs

30. IV site ke liye kaunsa arm avoid NAHI karna?
(a) Mastectomy side (b) Non-dominant healthy arm (c) AV fistula wala (d) Infected/ oedematous arm

31. Match: P. 14 G Q. 18 G R. 22 G S. 24 G — colour: 1. Yellow 2. Blue 3. Green 4. Orange
(a) P-4, Q-3, R-2, S-1 (b) P-3, Q-4, R-1, S-2 (c) P-4, Q-2, R-3, S-1 (d) P-1, Q-3, R-2, S-4

32. Assertion (A): Blood ke saath dextrose nahi chalate. Reason (R): Dextrose RBC haemolysis/ clumping karta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

33. Head injury patient me kaunsa fluid avoid?
(a) Mannitol (b) D5W (c) 0.9 % NS (d) 3 % NaCl

34. Most common transfusion reaction –
(a) Acute haemolytic (b) Anaphylaxis (c) Febrile non-haemolytic (d) Bacterial

✓ Answer Key & Explanation

1-b ~60 %.
2-a K^+ ICF, Na^+ ECF.
3-b 1 kg = 1 L.
4-d Sirf 0.9 % NS.

5-c 0.45 % hypotonic.
6-b 18 G green.
7-d 20 G pink.
8-c 24 G yellow.
9-d Kam number = bada diameter.
10-d Micro-drip 60.
11-b 2–6 °C.
12-c Room temp + agitation, 5 days.
13-b 4 hrs.
14-a O −ve.
15-a 245 mOsm/L.
16-c Infiltration – cool, pale, swollen.
17-a Stop, remove, elevate.
18-a Phlebitis – red, warm, tender.
19-a Air embolism – left lateral head down.
20-b Fluid overload.
21-a Acute haemolytic suspicion – stop.
22-d Naye tubing se – purana blood andar nahi jaye.
23-b Reactions first 15 min me hoti hain.
24-b Distal to proximal.
25-b 1000 × 15 ÷ 480 = 31.25.
26-c 500 × 20 ÷ 240 = 41.6.
27-b 1500 ÷ 12 = 125.
28-b Micro-drip me drops/min = ml/hr.
29-c 25 ÷ 15 = 1.67 ml/min → 1000 ÷ 1.67 ≈ 600 min = 10 hrs.
30-b Non-dominant healthy arm preferred hai.
31-a 14 orange, 18 green, 22 blue, 24 yellow.
32-a Dono sahi, R reason.
33-b D5W free water → cerebral oedema.
34-c Febrile non-haemolytic most common.

16 Medication Administration & Drug Calculations

Level 1 – Direct

1. "a.c." ka matlab –
   (a) Before meals (b) After meals (c) Immediately (d) At bedtime
2. "h.s." –
   (a) Hospital stay (b) Every hour (c) Half strength (d) At bedtime
3. "stat" –
   (a) Immediately (b) Daily (c) As needed (d) Statistics
4. "p.c." –
   (a) Per cent (b) After meals (c) Before meals (d) By mouth
5. Intradermal injection ka angle –
   (a) 10–15° (b) 45° (c) 90° (d) 30°
6. Subcutaneous injection ka angle –
   (a) 10° (b) 45° (c) 90° only (d) 180°
7. IM injection angle –
   (a) 15° (b) 45° (c) 90° (d) 25°
8. Sabse safe IM site –
   (a) Ventrogluteal (b) Dorsogluteal (c) Rectus femoris (d) Deltoid
9. Infant (< 1 yr) ke liye IM site –
   (a) Deltoid (b) Dorsogluteal (c) Vastus lateralis (d) Ventrogluteal only
10. Deltoid me max volume –
    (a) 1 ml (b) 3 ml (c) 5 ml (d) 0.1 ml
11. Z-track kis drug ke liye?
    (a) Iron dextran (b) Heparin (c) Vitamin B12 (d) Insulin
12. Mantoux test kab padhte hain?
    (a) 6 hrs (b) 24 hrs (c) 48–72 hrs (d) 7 days
13. Insulin syringe U-100 ka matlab –
    (a) 100 units/ml (b) 100 mg/ml (c) 100 ml (d) 10 units/ml
14. Kaunsa insulin IV de sakte hain?
    (a) NPH (b) Regular (c) Glargine (d) Detemir
15. Fastest absorption route (parenteral) –
    (a) SC (b) IV (c) IM (d) ID
16. 1 teaspoon =
    (a) 2 ml (b) 5 ml (c) 15 ml (d) 30 ml

Level 2 – Priority & Application

17. Dawa dene se pehle patient ko identify karne ka sahi tareeka –

(a) Bed number (b) Diagnosis (c) Face dekh ke (d) Patient se naam & DOB batwana + ID band

18. Insulin mix karna hai (Regular + NPH). Pehle syringe me –
(a) Koi bhi (b) Dono saath vial me (c) NPH (d) Regular (clear)

19. Medication error ho gaya. Nurse ka pehla kadam –
(a) Chhupana (b) Patient ko assess karna (c) Incident report (d) Doctor ko kal batana

20. Eye drops kahan daalein?
(a) Upper lid (b) Cornea par (c) Inner canthus par (d) Lower conjunctival sac

21. Eye drop ke baad nasolacrimal duct par pressure kyun?
(a) Koi faayda nahi (b) Aankh band (c) Systemic absorption kam (d) Dard kam

22. Adult me ear drops – pinna –
(a) Straight (b) Forward (c) Up & back (d) Down & back

23. 2 saal ke bachche me ear drops – pinna –
(a) Up & back (b) Down & back (c) Up (d) Forward

24. Enteric-coated tablet nigal nahi pa raha patient. Nurse –
(a) Doctor se doosra form maange (b) Crush karke de (c) Skip kare (d) Paani me ghol de

25. Patient dawa lene se mana karta hai. Nurse –
(a) Chhod de bina likhe (b) Zabardasti de (c) Reason pooche, samjhaye, refuse document kare, doctor ko inform (d) Khane me mila de

26. Heparin SC injection ke baad –
(a) Massage nahi, aspirate nahi (b) Massage karo (c) Garam pack (d) IM jaise

27. Narcotic drugs ka count –
(a) Kabhi nahi (b) Pharmacist hi (c) Har shift change par 2 nurses (d) Mahine me ek baar

? Level 3 – Advanced (Calculation / NOT / Match / Assertion-Reason)

28. Order: 500 mg; available: 250 mg tablet. Kitni tablets?
(a) 1 (b) 2 (c) 2.5 (d) 4

29. Order: 75 mg; available: 50 mg/2 ml. Kitne ml?
(a) 1.5 ml (b) 2 ml (c) 3 ml (d) 4 ml

30. Gentamicin 60 mg ordered; vial 80 mg/2 ml. Kitne ml?
(a) 0.75 (b) 1.5 (c) 2 (d) 2.5

31. Young's rule: adult dose 500 mg, child 6 yrs – child dose?
(a) 100 mg (b) 166.6 mg (c) 250 mg (d) 300 mg

32. Heparin 5000 units ordered; vial 25,000 units/5 ml. Kitne ml?
(a) 0.5 (b) 1 (c) 2 (d) 5

33. 30 units insulin U-100 syringe me kitne ml?
(a) 0.03 (b) 0.3 (c) 3 (d) 30

34. Adrenaline 1 : 1000 ka matlab –
(a) 1 mg/ml (b) 10 mg/ml (c) 0.1 mg/ml (d) 1 g/ml

35. Kaunsa tablet crush kar sakte hain?
(a) Sustained-release (b) Enteric-coated (c) Sublingual NTG (d) Plain paracetamol

36. Match: P. ID Q. SC R. IM S. IV — angle: 1. 90° 2. 25° 3. 10–15° 4. 45°
(a) P-2, Q-4, R-1, S-3 (b) P-3, Q-4, R-1, S-2 (c) P-4, Q-3, R-2, S-1 (d) P-3, Q-1, R-4, S-2

37. Assertion (A): Dorsogluteal site ab avoid ki jati hai. Reason (R): Sciatic nerve injury ka khatra hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

38. Medication ke 5 rights me kaunsa NAHI hai?
(a) Right route (b) Right dose (c) Right doctor (d) Right patient

39. "Do Not Use" abbreviation –
(a) U (units) (b) tab (c) mg (d) ml

40. Trough level kab lete hain?
(a) Dose ke turant baad (b) Next dose se thik pehle (c) 1 hr baad (d) Kabhi bhi

Answer Key & Explanation

1-a Ante cibum = before meals.
2-d Hora somni = bedtime.
3-a Statim = immediately.
4-b Post cibum = after meals.
5-a 10–15°, bevel up.
6-b 45° (obese 90°).
7-c 90°.
8-a Ventrogluteal – nerves/ vessels nahi.
9-c Vastus lateralis.
10-a Deltoid ≤ 1 ml.
11-a Iron dextran – staining & irritation.
12-c 48–72 hrs, induration.
13-a 100 units per ml.
14-b Regular (clear) hi IV.
15-b IV – 100 % bioavailability.
16-b 5 ml.
17-d 2 identifiers, open question.
18-d Clear before cloudy.
19-b Patient safety pehle.
20-d Lower conjunctival sac.
21-c Systemic absorption ↓.
22-c Adult up & back.
23-b < 3 yrs down & back.
24-a EC crush nahi – alternative form.
25-c Right to refuse respect + document.
26-a Haematoma rokne ke liye.
27-c Har shift, 2 nurses.
28-b 500/250 = 2.
29-c 75/50 × 2 = 3 ml.
30-b 60/80 × 2 = 1.5 ml.
31-b 6/18 × 500 = 166.6 mg.
32-b 5000/25000 × 5 = 1 ml.
33-b 30/100 = 0.3 ml.
34-a 1 g/1000 ml = 1 mg/ml.
35-d Plain tablet crush ho sakta hai.
36-b ID 10–15, SC 45, IM 90, IV 25.
37-a Dono sahi, R reason.
38-c Doctor rights me nahi.
39-a "U" galat padha ja sakta hai.
40-b Trough = lowest – next dose se pehle.

17 Hot & Cold, Wound Care, Bandaging & Specimens

Level 1 – Direct

1. Heat application se blood vessels –
   (a) Rupture (b) Dilate (c) Constrict (d) No change
2. Sitz bath ka temperature –
   (a) 20 °C (b) 40–43 °C (c) 55 °C (d) 60 °C
3. Sitz bath ka time –
   (a) 2 min (b) 15–20 min (c) 1 hr (d) 2 hrs
4. Tepid sponge ka paani –
   (a) 10 °C (b) 27–34 °C (c) 45 °C (d) 50 °C
5. Granulation tissue ka rang –
   (a) Beefy red (b) Black (c) Yellow (d) White
6. Pus wali drainage –
   (a) Serosanguineous (b) Serous (c) Purulent (d) Sanguineous
7. Face ke sutures kab nikalte hain?
   (a) 3–5 days (b) 7–10 days (c) 14 days (d) 21 days
8. Joints (elbow/ knee) par kaunsa bandage turn?
   (a) Spiral (b) Circular (c) Recurrent (d) Figure of 8
9. Stump/ head par bandage turn –
   (a) Figure of 8 (b) Spica (c) Spiral (d) Recurrent
10. Forearm (cone-shaped) par –
    (a) Reverse spiral (b) Recurrent (c) Spica (d) Circular
11. Perineal dressing hold karne ke liye –
    (a) Crepe (b) T-binder (c) Sling (d) Many-tailed binder
12. Wound healing ka sabse important nutrient –
    (a) Carbohydrate (b) Protein (c) Fibre (d) Fat
13. Blood culture kab lena chahiye?
    (a) Antibiotic ke baad (b) Khane ke baad (c) Kabhi bhi (d) Antibiotic se pehle, 2 sites
14. Sputum for AFB –
    (a) Saliva (b) Early morning, plain water se kulla ke (c) Toothpaste ke baad (d) Shaam

Level 2 – Priority & Application

15. Ankle sprain hua 2 ghante pehle. Kya lagaye?
    (a) Massage (b) Hot fomentation (c) Hot water bag (d) Ice pack
16. Abdominal surgery ke baad patient bolta hai "kuch phat gaya" aur wound se intestine dikh rahi hai. Pehla action –
    (a) Andar dabao (b) Patient ko chalao (c) Dry gauze (d) Sterile saline-soaked dressing se dhako, low Fowler's knees flexed, help bulao

17. Wound cleaning ki direction –
(a) Bahar se andar (b) Circular dirty se clean (c) Incision (clean) se bahar ki taraf (d) Koi bhi

18. Drain site kab saaf karein?
(a) Sabse pehle (b) Kabhi nahi (c) Incision ke baad, alag swab se (d) Incision ke saath ek hi swab se

19. Bandage ke baad patient ki ungliyan thandi aur sunn ho gayi. Nurse –
(a) Aur tight kare (b) Bandage turant dheela kare (c) Ignore (d) Ice lagaye

20. Tepid sponge ke time patient kaanpne laga. Nurse –
(a) Ice pack (b) Fan tez (c) Thanda paani aur (d) Sponge rok de

21. Wound swab lene se pehle –
(a) Antiseptic laga ke (b) NS se wound saaf karke viable tissue se (c) Dressing ke upar se (d) Pus se swab

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

22. Heat kis condition me contraindicated NAHI hai?
(a) Injury ke pehle 24 hrs (b) Chronic joint stiffness (c) Active bleeding (d) Acute appendicitis

23. Cold application kis me contraindicated?
(a) Fresh sprain (b) Peripheral vascular disease (c) Post-tonsillectomy (d) Epistaxis

24. Match: P. Primary intention Q. Secondary intention R. Tertiary intention S. Dehiscence — 1. Wound open chhoda, granulation se bharta 2. Edges sutured 3. Delayed closure 4. Wound edges ka khulna
(a) P-3, Q-1, R-2, S-4 (b) P-2, Q-3, R-1, S-4 (c) P-1, Q-2, R-4, S-3 (d) P-2, Q-1, R-3, S-4

25. Assertion (A): Heat 45–60 min se zyada lagane par faayda ulta ho jata hai. Reason (R): Rebound phenomenon me vasoconstriction hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

26. Bandaging ka kaunsa rule GALAT hai?
(a) Distal se proximal (b) Fingertips dhak do (c) Circular turns se shuru & khatam (d) Har turn pichle ka ⅔ cover kare

27. Heavy exudate wale wound ke liye best dressing –
(a) Dry gauze only (b) Hydrogel (c) Transparent film (d) Alginate

28. Dry wound me moisture add karne wali dressing –
(a) Alginate (b) Gauze (c) Foam (d) Hydrogel

29. Blood sample tubes ka order of draw – pehle –
(a) EDTA (b) Plain (c) Fluoride (d) Blood culture

30. Jackson-Pratt drain me suction kaise banta hai?
(a) Pump (b) Bulb compress karke cap band (c) Wall suction (d) Gravity

Answer Key & Explanation

1-b Heat = vasodilation.
2-b 40–43 °C.
3-b 15–20 min.
4-b 27–34 °C.
5-a Beefy red = healthy granulation.
6-c Purulent = pus.
7-a Face 3–5 days.
8-d Figure of 8.
9-d Recurrent.
10-a Reverse spiral.
11-b T-binder.
12-b Protein.

13-d Antibiotics se pehle, 2 sets.
14-b Early morning deep cough.
15-d Pehle 24–48 hrs cold.
16-d Evisceration emergency.
17-c Clean to dirty.
18-c Drain last, separately.
19-b Circulation compromised.
20-d Shivering heat banata hai.
21-b Levine technique.
22-b Chronic stiffness me heat faydemand.
23-b PVD me circulation aur ghatega.
24-d Primary sutured, secondary open, tertiary delayed, dehiscence opening.
25-a Dono sahi, R reason.
26-b Fingertips khule rakho (CSM check).
27-d Alginate absorbent.
28-d Hydrogel.
29-d Blood culture pehle (sterility).
30-b Compress & cap.

18 First Aid & Emergency Care

Level 1 – Direct

1. First aid ke 3 P's me kaunsa hai?
(a) Perform surgery (b) Pay bill (c) Preserve life (d) Prescribe drugs

2. Adult CPR compression rate –
(a) 60–80 (b) 100–120 (c) 140–160 (d) 80–100

3. Adult CPR depth –
(a) 2–3 cm (b) 5–6 cm (c) 8–10 cm (d) 1 cm

4. Adult CPR ratio (1 rescuer) –
(a) 15 : 2 (b) 30 : 2 (c) 5 : 1 (d) 30 : 1

5. Infant CPR me pulse kahan check?
(a) Femoral (b) Radial (c) Brachial (d) Carotid

6. Pulse check ka max time –
(a) 5 sec (b) 10 sec (c) 30 sec (d) 1 min

7. Arterial bleeding –
(a) Bright red, spurting (b) Oozing (c) Black (d) Dark red, steady

8. Bleeding control ka pehla method –
(a) Pressure point (b) Ice (c) Direct pressure (d) Tourniquet

9. Shock me position –
(a) Flat with legs raised (b) Sitting (c) Prone (d) Head up

10. Nose bleed me position –
(a) Lie flat (b) Sit, lean forward (c) Head between knees lying (d) Head back

11. Unconscious but breathing patient –
(a) Supine (b) Prone flat (c) Recovery (lateral) position (d) Sitting

12. Triage me red tag –
(a) Delayed (b) Dead (c) Minor (d) Immediate

13. India me ambulance number –
(a) 100 (b) 101 (c) 108 (d) 1098

14. Burn par first aid –
(a) Ice (b) Ghee/ butter (c) Toothpaste (d) Cool running water 20 min

Level 2 – Priority & Application

15. Accident scene par first aider ka pehla kadam –
(a) Photo lena (b) Victim ko uthana (c) Paani pilana (d) Scene safety (danger) check

16. Conscious adult choke ho raha hai, bol nahi pa raha. Action –
(a) Finger sweep blind (b) Abdominal thrusts (Heimlich) (c) Paani pilao (d) Peeth thapthapao only

17. Infant choking – action –
(a) Ulta latka ke hilao (b) Abdominal thrusts (c) 5 back blows + 5 chest thrusts (d) Blind finger sweep

18. Pregnant woman choking –
(a) Chest thrusts (b) Kuch nahi (c) Abdominal thrusts (d) Back blows only

19. Fracture ko splint karte waqt –
(a) Tight bandage (b) Sirf fracture site (c) Bone seedhi karo pehle (d) Joint above & below include

20. Open fracture me bone bahar hai –
(a) Paani se dhoke andar (b) Andar dabao (c) Sterile dressing se dhako, push mat karo, immobilize (d) Kuch nahi

21. Electric shock victim – pehla kadam –
(a) Turant haath se kheencho (b) Paani dalo (c) Power switch off / dry non-conducting object se alag karo (d) CPR without separating

22. Kerosene pi gaya bachcha –
(a) Ulti karwao (b) Namak paani (c) Milk se ulti (d) Ulti NAHI karwani, hospital le jao

23. Snake bite me kya NAHI karna?
(a) Immobilize (b) Hospital le jana (c) Tourniquet lagana, cut karna (d) Reassure

24. Dog bite wound ka pehla care –
(a) Mirchi lagana (b) Turant suture (c) Soap & running water se 15 min dhona (d) Bandage tight

25. Heat stroke victim (40.5 °C, confused) –
(a) Garam chai (b) Rapid cooling (c) Blanket (d) Paracetamol only

26. Seizure ke dauran –
(a) Paani pilao (b) Haath pair pakdo (c) Head protect karo, restrain mat karo (d) Mouth me spoon

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

27. Burn first aid me kya NAHI karna chahiye?
(a) Jewellery nikalna (b) Clean cloth se dhakna (c) Cool water 20 min (d) Blister phodna

28. Match: P. Red Q. Yellow R. Green S. Black — 1. Dead/ expectant 2. Walking wounded 3. Delayed 4. Immediate
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-2, R-3, S-1 (c) P-4, Q-3, R-2, S-1 (d) P-3, Q-4, R-1, S-2

29. Assertion (A): Infant choking me abdominal thrusts nahi dete. Reason (R): Infant ke liver ko injury ho sakti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Child/ infant CPR – 2 rescuers ratio –
(a) 30 : 2 (b) 15 : 2 (c) 5 : 1 (d) 3 : 1

31. Tourniquet ke baare me GALAT statement –
(a) Life-threatening limb bleeding me (b) Wound se 5–7.5 cm upar (c) Har 2 min dheela karna (d) Application time note karna

32. Shock ka earliest sign –
(a) Coma (b) Restlessness/ anxiety (c) Hypotension (d) Bradycardia

33. Bee sting nikalne ka sahi tareeka –
(a) Card se scrape karna (b) Chhod do (c) Daanton se (d) Tweezers se dabana

34. Lone rescuer, infant arrest, phone nahi – pehle –
(a) Wait karo (b) Sirf breaths (c) Call karo (d) 2 min CPR phir call

Answer Key & Explanation

1-c Preserve life, prevent deterioration, promote recovery.
2-b 100–120/min.
3-b 5–6 cm.
4-b 30 : 2.
5-c Brachial.
6-b ≤ 10 sec.
7-a Bright red, pulsatile.
8-c Direct pressure.
9-a Legs raised.
10-b Lean forward – blood nigla na jaye.
11-c Recovery position.
12-d Red = immediate.
13-c 108.
14-d Cool running water.
15-d Self-safety pehle.
16-b Heimlich.
17-c Back blows + chest thrusts.
18-a Chest thrusts.
19-d Joint above & below.
20-c Push nahi, sterile cover.
21-c Pehle current se alag.
22-d Aspiration pneumonitis risk.
23-c Tourniquet/ cut/ suck nahi.
24-c Soap & water 15 min.
25-b Rapid cooling.
26-c Protect, no restraint, nothing in mouth.
27-d Blister phodne se infection.
28-c Red immediate, yellow delayed, green minor, black dead.
29-a Dono sahi, R reason.
30-b 15 : 2.
31-c First aider tourniquet dheela nahi karta.
32-b Restlessness earliest.
33-a Squeeze se venom inject hota hai.
34-d Child/ infant – 2 min CPR first.

19 Unconscious Patient, Peri-operative, Stress, Grief & Death

Level 1 – Direct

1. Kübler-Ross ka first stage –
   (a) Denial (b) Anger (c) Acceptance (d) Bargaining
2. Kübler-Ross ka last stage –
   (a) Acceptance (b) Anger (c) Denial (d) Depression
3. "Bas beti ki shaadi tak jee lun" – kaunsa stage?
   (a) Anger (b) Denial (c) Bargaining (d) Depression
4. Dying patient me sabse last kaunsa sense jata hai?
   (a) Vision (b) Smell (c) Taste (d) Hearing
5. Death ke baad body ka thanda hona –
   (a) Algor mortis (b) Pallor mortis (c) Rigor mortis (d) Livor mortis
6. Death ke baad dependent parts me purple discolouration –
   (a) Livor mortis (b) Rigor (c) Putrefaction (d) Algor
7. Rigor mortis kab shuru hota hai?
   (a) 10 min (b) 2–4 hrs (c) 24 hrs (d) 72 hrs
8. Rigor mortis pehle kahan dikhta hai?
   (a) Jaw/ eyelids (small muscles) (b) Arms (c) Abdomen (d) Legs
9. General Adaptation Syndrome kisne diya?
   (a) Maslow (b) Freud (c) Hans Selye (d) Erikson
10. GAS ka first stage –
    (a) Resistance (b) Recovery (c) Exhaustion (d) Alarm
11. Eye donation death ke kitne ghante ke andar?
    (a) 6 hrs (b) 24 hrs (c) 48 hrs (d) 72 hrs
12. Healthiest defence mechanism –
    (a) Projection (b) Sublimation (c) Regression (d) Denial
13. Pre-op NPO (solids) –
    (a) 2 hrs (b) 6–8 hrs (c) 24 hrs (d) 30 min

Level 2 – Priority & Application

14. Unconscious patient ki care me sabse pehli priority –
    (a) Hair care (b) Nutrition (c) Airway maintain karna (d) Bed bath
15. Unconscious patient ki aankhen –
    (a) Lubricant, eye pads/ lids tape – corneal ulcer prevention (b) Khuli chhod do (c) Kuch nahi (d) Ice
16. Dying patient gussa karta hai aur nurse ko daantta hai. Nurse –
    (a) Room chhod de (b) Wapas jawab de (c) Personally na le, sune aur support kare (d) Sedative de

17. Dead body me dentures –
(a) Wapas lagao (face shape maintain) (b) Nikal ke phenk do (c) Pet me rakho (d) Relatives ko do turant

18. Medico-legal case me death ke baad tubes/ lines –
(a) Sirf IV nikalo (b) Sab nikal do (c) Relatives decide karein (d) Nahi nikalni, police ko inform

19. Dead body ke sir ke neeche pillow kyun?
(a) Rigor rokne ke liye (b) Comfort (c) Face me livor mortis (discolouration) rokne ke liye (d) Tradition

20. Pre-medication dene ke baad –
(a) Khana do (b) Patient ko chalne do (c) Side rails up, bed par rakho (d) Bathroom bhejo

21. Post-op patient (general anaesthesia) gag reflex aane tak position –
(a) Lateral/ side-lying (b) Supine flat (c) Prone (d) Sitting

? Level 3 – Advanced (NOT / Match / Assertion-Reason)

22. Approaching death ka sign kaunsa NAHI hai?
(a) Rising BP & strong pulse (b) Decreased urine output (c) Mottled cold extremities (d) Cheyne-Stokes breathing

23. Last office me kya GALAT hai?
(a) ID tags wrist & toe par (b) MLC me tubes nikal dena (c) Body supine, pillow under head (d) Aankhen band karna

24. Match: P. Denial Q. Anger R. Bargaining S. Acceptance — 1. "Mere saath hi kyun?" 2. "Ye report galat hai" 3. Calm, planning 4. "Agar main theek ho gaya to..."
(a) P-2, Q-1, R-4, S-3 (b) P-3, Q-1, R-4, S-2 (c) P-2, Q-4, R-1, S-3 (d) P-1, Q-2, R-3, S-4

25. Assertion (A): Dying patient ke paas bolte waqt dhyan rakhna chahiye. Reason (R): Hearing sabse last me jaata hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

26. Brain death certify karne ke liye India me (THOTA) –
(a) Board of 4 doctors (b) Nurse (c) Police (d) 1 doctor

27. Repression vs suppression –
(a) Same (b) Dono conscious (c) Repression unconscious, suppression conscious (d) Repression conscious

28. Organ donation (heart, liver) kis donor se?
(a) Brain-dead (heart-beating) donor (b) Sirf living (c) Kisi se bhi (d) Cardiac death after 24 hrs

✓ Answer Key & Explanation

1-a Denial pehla.
2-a Acceptance last.
3-c Bargaining – deal.
4-d Hearing last.
5-a Algor = cooling.
6-a Livor = lividity.
7-b 2–4 hrs.
8-a Small muscles pehle.
9-c Hans Selye.
10-d Alarm reaction.
11-a 6 hrs ke andar.
12-b Sublimation – socially acceptable.
13-b 6–8 hrs solids, 2 hrs clear fluids.
14-c Airway first.
15-a No blink reflex – cornea protect.
16-c Anger stage – support.
17-a Face shape ke liye.
18-d MLC – evidence preserve.
19-c Blood face me pool na ho.
20-c Fall risk.
21-a Aspiration prevention.
22-a Death ke paas BP girta hai, pulse weak.

23-b MLC me tubes nahi nikalte.
24-a Denial "report galat", anger "kyun", bargaining "agar", acceptance calm.
25-a Dono sahi, R reason.
26-a 4-doctor panel.
27-c Repression unconscious, suppression conscious.
28-a Brain-dead heart-beating donor.

20 GSSSB Pattern Mock Test – 100 MCQs (Full Syllabus)



Mock test ke rules

100 questions · 100 marks · 75 minutes (Fundamentals section ka practice). Galat answer par **−0.25**. Timer lagao, OMR jaise alag kagaz par answer likho, phir answer key se milao.
Score guide: 80+ Excellent · 65–79 Good · 50–64 Weak topics revise karo · < 50 Notes dobara padho.

? Mock Test – Part A (1–50)

1. Florence Nightingale ko "Lady with the Lamp" kis war me kaha gaya?
(a) Boer War (b) Crimean War (c) World War I (d) World War II

2. Indian Nursing Council ek ___ body hai.
(a) Private (b) International (c) Voluntary (d) Statutory

3. Patient ki privacy aur information gupt rakhna –
(a) Justice (b) Veracity (c) Confidentiality (d) Fidelity

4. Doctor ka verbal order sirf kab lena chahiye?
(a) Hamesha (b) Kabhi nahi (c) Emergency me, read-back ke saath (d) Night shift me

5. Health ki WHO definition me kaunsa dimension NAHI hai?
(a) Economic (b) Physical (c) Mental (d) Social

6. Nursing diagnosis ka "label" kis organization ki list se?
(a) NANDA-I (b) INC (c) ICN (d) WHO

7. Kaunsa patient sabse pehle dekhna chahiye?
(a) Asthma patient with wheeze & SpO_2 86 % (b) Post-op patient pain 5/10 (c) Patient discharge chahta hai (d) Diabetic sugar 180

8. Goal "Patient will ambulate 20 m with walker by day 2" me "by day 2" –
(a) Subject (b) Criteria (c) Time frame (d) Verb

9. 37.8 °C = ? °F
(a) 98.6 (b) 100 (c) 101 (d) 102

10. Oral temperature kis patient me nahi lena chahiye?
(a) Post-op day 3 alert (b) Unconscious patient (c) Teenager (d) Alert adult

11. BP cuff bahut bada hai – reading –
(a) Falsely low (b) Accurate (c) Zero (d) Falsely high

12. Respiration sabse accurately kaise count karein?
(a) 10 sec × 6 (b) Patient se poochke (c) Pulse lene ke bahane, 1 full minute (if irregular) (d) Patient ko batake

13. Bradycardia –
(a) < 60/min (b) > 100/min (c) < 12/min (d) > 20/min

14. Over-bed table par jhuk ke saans lena –
(a) Supine (b) Orthopnoeic (c) Sims' (d) Fowler's

15. Tympanic thermometer – 2 saal ka bachcha – pinna –
(a) Forward (b) Up & forward (c) Up & back (d) Down & back

16. Physical exam me palpation ke liye temperature –
(a) Palm (b) Dorsum of hand (c) Thumb (d) Fingertips

17. Stomach par percussion –
(a) Tympanic (b) Flat (c) Dull (d) Resonant

18. Heart sound S1 kis valves ke band hone se?
(a) Only mitral (b) Aortic & pulmonary (c) Mitral & tricuspid (d) Only aortic

19. Infection ke chain me "fomites" kis mode me aate hain?
(a) Droplet (b) Indirect contact (c) Vector (d) Airborne

20. Chickenpox patient – isolation –
(a) Airborne + contact (b) Droplet (c) Protective (d) Contact only

21. Sterile gloves pehnte waqt pehla glove kahan se pakdein?
(a) Cuff ke andar (inner) se (b) Fingers se (c) Bahar ki surface se (d) Kahin se bhi

22. Ethylene oxide sterilization ke baad aeration kyun?
(a) Smell (b) Toxic gas residue hatane (c) Dry karne (d) Colour

23. Autoclave indicator tape par sterilization ke baad –
(a) Black/ brown stripes aate hain (b) Red ho jata hai (c) Tape gir jata hai (d) Koi change nahi

24. Expired medicines kis BMW category me?
(a) Red (b) Blue (c) White (d) Yellow

25. Bed bath me sabse last kya saaf karte hain?
(a) Perineum (b) Face (c) Arms (d) Legs

26. Denture storage –
(a) Labelled cup me paani ke saath (b) Pillow ke neeche (c) Khule me (d) Tissue me lapet ke

27. Pediculosis capitis –
(a) Pubic lice (b) Body lice (c) Head lice (d) Scabies

28. Occupied bed kab banate hain?
(a) Discharge ke baad (b) Patient bed se utar na sake (c) Admission se pehle (d) Patient ambulatory ho

29. Bed cradle kis patient me?
(a) Pneumonia (b) Hypertension (c) Diabetes (d) Burns/ gout/ fracture

30. REM sleep ka feature –
(a) Dreaming, eye movements (b) Muscle tone high (c) Deepest sleep (d) Growth hormone

31. Opioid overdose me pupils –
(a) Pinpoint (b) Dilated (c) Normal (d) Unequal

32. TENS kis theory par based?
(a) Gate control (b) Stress (c) Pattern (d) Specificity

33. Pain ke liye non-pharmacological measure –
(a) Paracetamol (b) Distraction & massage (c) Morphine (d) Ibuprofen

34. Plantar flexion contracture –
(a) Wrist drop (b) Footdrop (c) Claw hand (d) Hip flexion

35. Log rolling kitne logon se?
(a) 1 (b) 3–4 (c) 6 (d) 10

36. Crutch walking me "3-point gait" kab?
(a) Dono legs partial weight (b) Ek leg non-weight bearing (c) Healthy person (d) Paraplegia

37. Braden scale ka score range –
(a) 0–10 (b) 6–23 (c) 5–20 (d) 1–100

38. Stage 3 pressure ulcer –
(a) Redness only (b) Blister (c) Bone visible (d) Full thickness, fat visible

39. HOB 30° se zyada rakhne par kaunsa force badhta hai?
(a) Gravity only (b) Friction (c) Shear (d) Torque

40. Vitamin K ki kami –
(a) Scurvy (b) Night blindness (c) Rickets (d) Bleeding

41. Iodine deficiency –
(a) Goitre (b) Anaemia (c) Pellagra (d) Scurvy

42. Gluten-free diet kis disease me?
(a) Diabetes (b) Gout (c) Celiac disease (d) Hypertension

43. Low-residue diet kab?
(a) Obesity (b) Constipation (c) Diverticulosis (d) Diarrhoea/ before colonoscopy

44. NG tube medicines dene ke beech flush –
(a) Har dawa ke beech water (b) Sirf end me (c) Milk se (d) Nahi

45. TPN line se kya nahi dena chahiye?
(a) Doosri dawa/ blood (b) TPN (c) Kuch bhi de sakte (d) Saline

46. Urine ka specific gravity badhta hai –
(a) Diabetes insipidus (b) Renal failure (c) Overhydration (d) Dehydration

47. Clean-catch midstream urine me –
(a) Last part collect (b) Bag se (c) Pehla urine collect (d) Pehla part toilet me, beech ka collect

48. Indwelling catheter ko kya kehte hain?
(a) Condom (b) Nelaton (c) Robinson (d) Foley

49. Condom catheter ke tip par space –
(a) 2.5–5 cm (b) 10 cm (c) Nahi (d) 15 cm

50. Urinary retention ke liye Credé's manoeuvre –
(a) Suprapubic pressure (b) Enema (c) Catheter (d) Kegel

? Mock Test – Part B (51–100)

51. Oil retention enema kitni der retain?
(a) 5 min (b) 30 min – 3 hrs (c) 12 hrs (d) 24 hrs

52. Child (5–12 yrs) cleansing enema volume –
(a) 50 ml (b) 300–500 ml (c) 1000 ml (d) 1500 ml

53. Suppository adult me kitna andar?
(a) 1 cm (b) 7.5–10 cm (c) 15 cm (d) 20 cm

54. Meconium kis rang ka hota hai?
(a) Brown (b) Greenish-black (c) Yellow (d) Clay

55. Steatorrhoea –
(a) Blood in stool (b) Black stool (c) Fatty, frothy, floating stool (d) Rice-water stool

56. Venturi mask ka main advantage –
(a) Exact FiO_2 (b) Cheap (c) Comfortable eating (d) High flow

57. Non-rebreather mask me reservoir bag –
(a) Paani se bharein (b) Collapse nahi hona chahiye (c) Hata dein (d) Khaali rehna chahiye

58. O_2 toxicity kab?
(a) Kabhi nahi (b) Nasal cannula 2 L (c) FiO_2 21 % (d) FiO_2 > 50–60 % for > 24–48 hrs

59. Steam inhalation me kaunsa medicine?
(a) Tincture benzoin (b) Insulin (c) Digoxin (d) Heparin

60. Nebulized salbutamol ka side effect –
(a) Hypotension (b) Bradycardia (c) Tachycardia & tremors (d) Constipation

61. Suction catheter size – ET tube ke internal diameter ka –
(a) Same (b) Teen guna (c) Double (d) Half se kam ya barabar

62. Postural drainage – lower lobes –
(a) Prone head up (b) Trendelenburg (head down) (c) Supine flat (d) Sitting upright

63. Percussion (clapping) chest par kaise?
(a) Fist (b) Fingers tips (c) Cupped hands, cloth ke upar (d) Flat palm

64. Dehydration ka sign –
(a) Bounding pulse (b) Crackles (c) Poor skin turgor, weak rapid pulse (d) Oedema

65. Fluid overload ka sign –
(a) JVD, crackles, bounding pulse (b) Poor turgor (c) Concentrated urine (d) Sunken eyes

66. Ringer's lactate kis condition me common?
(a) Blood transfusion line (b) Hypoglycaemia (c) Head injury D5W (d) Burns (Parkland)

67. IV tourniquet kitni der max?
(a) 1–2 min (b) 10 min (c) 30 min (d) 1 hr

68. Peripheral IV cannula kitne din me badlein?
(a) Daily (b) 7 days (c) 72–96 hrs (d) 14 days

69. 1 litre fluid 6 ghante me – ml/hr –
(a) 100 (b) 166.6 (c) 200 (d) 250

70. 600 ml 5 ghante me, DF 20 – drops/min –
(a) 30 (b) 40 (c) 50 (d) 60

71. Allergic transfusion reaction (urticaria) –
(a) Stop/ slow, antihistamine (b) Ignore (c) Transfusion fast (d) Adrenaline only

72. FFP storage –
(a) 2–6 °C (b) ≤ −18 °C / −30 °C (c) 20–24 °C (d) 37 °C

73. Drug ka "half-life" –
(a) Drug ka 50 % concentration girne ka time (b) Action shuru hone ka time (c) Dose ka half (d) Shelf life ka half

74. Sublingual nitroglycerin –
(a) Nigal lo (b) Paani ke saath (c) Jeebh ke neeche ghulne do (d) Chaba lo

75. Iron syrup kaise dein?
(a) Straw se (daant stain rokne) (b) Chai ke saath (c) Antacid ke saath (d) Milk ke saath

76. ID injection ke baad –
(a) Massage nahi, bleb banna chahiye (b) Ice (c) Bandage (d) Massage

77. Insulin ka fastest absorption site –
(a) Thigh (b) Buttock (c) Arm (d) Abdomen

78. Ventrogluteal site landmark –
(a) PSIS only (b) Knee (c) Acromion (d) Greater trochanter, ASIS, iliac crest

79. Vastus lateralis – thigh ka kaunsa hissa?
(a) Upper 1/3 (b) Middle 1/3 anterolateral (c) Lower 1/3 medial (d) Posterior

80. Eye ointment ki ribbon –
(a) Upar lid par (b) Cornea par (c) Inner to outer canthus (d) Outer to inner

81. Clark's rule kis par based?
(a) Weight (b) Age (c) Height (d) Sex

82. 1 grain = ?
(a) 6 mg (b) 60–65 mg (c) 600 mg (d) 1 g

83. Hot water bag me air kyun nikalein?
(a) Weight (b) Leak (c) Sound (d) Flexible & conform hone ke liye, heat transfer

84. Heat application – suppuration (pus localisation) –
(a) Ghatata hai (b) Rokta hai (c) Koi asar nahi (d) Badhata hai

85. Wound healing ka maturation phase –
(a) 0–4 days (b) 4–21 days (c) 21 days – 2 years (d) 1 hour

86. Abdominal sutures removal –
(a) 3–5 days (b) 7–10 days (c) 21 days (d) 30 days

87. Shoulder par bandage turn –
(a) Recurrent (b) Spiral (c) Spica (d) Circular

88. Triangular bandage ka common use –
(a) Abdomen (b) Arm sling (c) Head capeline (d) Perineum

89. Throat swab lete waqt kya touch nahi karna?
(a) Tonsils (b) Tongue/ cheeks (c) Swab stick (d) Posterior pharynx

90. CPR me hands kahan?
(a) Upper sternum (b) Xiphoid (c) Left chest (d) Lower half of sternum

91. Advanced airway ke saath ventilation rate –
(a) 1 breath every 6 sec (b) 1 every 2 sec (c) 30 : 2 (d) 1 every 15 sec

92. AED pads kahan?
(a) Back only (b) Abdomen (c) Dono left side (d) Right upper chest & left side below armpit

93. Fainting patient –
(a) Prone (b) Flat with legs raised (c) Sitting upright (d) Standing

94. Insect in ear –
(a) Kuch nahi (b) Pin se nikalo (c) Cotton bud (d) Warm oil/ water dalo (TM intact ho to)

95. Chemical eye burn –
(a) Turant 15–30 min paani se irrigation (b) Ointment (c) Antibiotics (d) Patch

96. Hypoglycaemic conscious patient –
(a) Insulin (b) NPO (c) 15 g sugar/ juice (d) Glucagon IV

97. Unconscious patient – kya nahi dena?
(a) Oxygen (b) Anything by mouth (c) Positioning (d) Suction

98. Kübler-Ross ka "depression" stage kaunsa number?
(a) 2 (b) 3 (c) 4 (d) 5

99. Algor mortis me body temperature girta hai lagbhag –
(a) 1 °C/hr (b) 10 °C/hr (c) Nahi girta (d) 5 °C/hr

100. Dead body par ID tags –
(a) Nahi lagate (b) Sirf shroud (c) Wrist, ankle/ toe & shroud (d) Sirf forehead

Answer Key – Mock Test (with explanation)

1-b Crimean War 1854–56 (Scutari).
2-d INC Act 1947 se bani statutory body.
3-c Confidentiality.
4-c Emergency + read-back + 24 hrs me sign.
5-a Economic WHO definition me nahi.
6-a NANDA-I taxonomy.
7-a Breathing problem + low SpO_2 – ABC.
8-c Time frame (T of SMART).
9-b 37.8 × 9/5 + 32 = 100 °F.
10-b Unconscious – oral contraindicated.
11-a Wide cuff = falsely low.
12-c Patient ko pata na chale.
13-a Pulse < 60.
14-b Orthopnoeic.
15-d < 3 yrs down & back.
16-b Dorsum.
17-a Tympanic.
18-c AV valves – S1.
19-b Fomites = indirect contact.
20-a Airborne + contact.
21-a Pehla glove skin-to-skin (inside cuff).
22-b ETO toxic residue.
23-a Stripes dark ho jaati hain.
24-d Yellow (discarded medicines).
25-a Perineum last.
26-a Labelled cup with water.
27-c Head lice.
28-b Patient bed-bound.
29-d Linen ka weight hataane ke liye.
30-a Dreaming & REM.
31-a Pinpoint pupils.
32-a Gate control.
33-b Distraction, massage.
34-b Footdrop.
35-b 3–4 persons.
36-b One leg non-weight bearing.
37-b 6–23.
38-d Fat visible.
39-c Shear.
40-d Clotting factors kam.
41-a Goitre.
42-c Celiac.
43-d Diarrhoea/ bowel prep.
44-a Har dawa ke beech flush.
45-a Dedicated line.
46-d Concentrated urine.
47-d Midstream collect.
48-d Foley – self-retaining balloon.
49-a 2.5–5 cm.
50-a Suprapubic pressure.
51-b 30 min – 3 hrs.
52-b 300–500 ml.
53-b 7.5–10 cm.
54-b Greenish-black, tarry.
55-c Fat malabsorption.
56-a Precise FiO_2.
57-b Bag collapse = O_2 kam.
58-d High FiO_2 prolonged.
59-a Benzoin (Friar's balsam).
60-c Beta-2 agonist – tachycardia.
61-d ≤ half internal diameter.
62-b Head down.
63-c Cupped hands over cloth.
64-c Poor turgor, weak pulse.
65-a JVD, crackles.
66-d Burns – Parkland formula me RL.
67-a 1–2 min.
68-c 72–96 hrs.
69-b 1000 ÷ 6 = 166.6.
70-b 600 × 20 ÷ 300 = 40.
71-a Antihistamine.
72-b ≤ −18 °C (1 yr).
73-a 50 % concentration fall.
74-c Sublingual.
75-a Straw – teeth stain.
76-a No massage, bleb.
77-d Abdomen.
78-d Trochanter, ASIS, iliac crest.
79-b Middle third anterolateral.
80-c Inner to outer.
81-a Weight (lb ÷ 150).
82-b 60–65 mg.

83-d Conform & heat transfer.
84-d Heat pus localize karta hai.
85-c Remodelling 21 days – 2 yrs.
86-b 7–10 days.
87-c Spica.
88-b Arm sling.
89-b Tongue/ cheeks contamination.
90-d Lower half of sternum.
91-a 10 per min.
92-d Right upper, left lateral.
93-b Legs raised.
94-d Warm oil (TM intact).
95-a Immediate irrigation.
96-c Rule of 15.
97-b Aspiration risk.
98-c Denial 1, anger 2, bargaining 3, depression 4, acceptance 5.
99-a ~1 °C/hr.
100-c Wrist, toe & shroud.

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